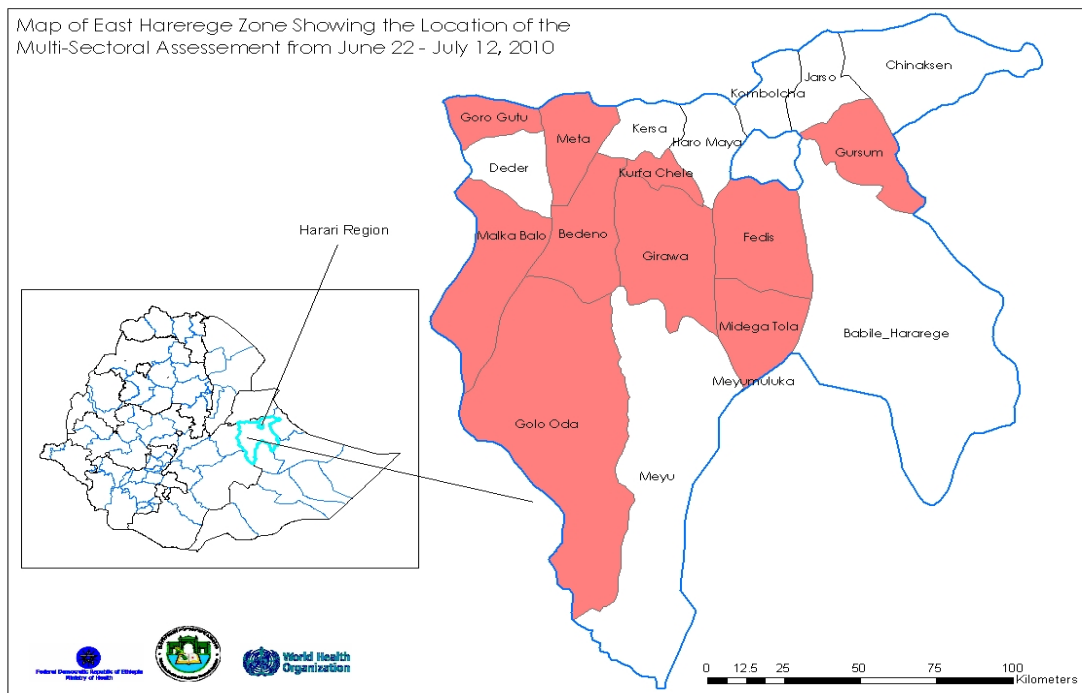


Technical Report

on:

“Belg Season” Multi – Sectoral Humanitarian needs Assessment in East and West Harerege Zones of Oromia Region Ethiopia



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June 22 – July 12, 2010



Photo Source: Negusu Worku (WHO/EHA). Team of East and West Harerege Zone. July, 2010

East and West Harerege Multi-Sectoral needs assessment team

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Mr. Reshid (USAID), and Mr. Getahun (East Zone DRMFSS).

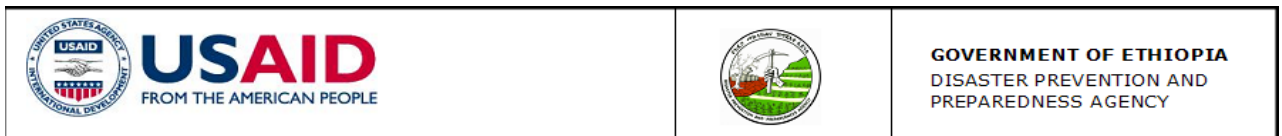


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Acronyms

AR	Attack Rate
ARI	Acute Respiratory Infections
AWD	Acute Watery Diarrhea
CBNP	Community Based Nutrition Program
CFR	Case Fatality Rate
DRMFSS	Disaster Risk Management and Food Security Sector
EHZ	East Harereg Zone
HEWs	Health Extension Workers
ITN	Insecticide Treated Net
IFHP	International Family Health Planning
ORHB	Oromia Regional Health Bureau
OTP	Outpatient Therapeutic Program
PHC	Primary Health Care
PHEM	Public Health Emergency Management
RDT	Rapid Diagnosis Test
RHB	Regional Health Bureau
SAM	Sever Acute Malnourished
SC	Stabilization Center
UNICEF	United Nations Children’s Fund
UTI	Urinary Tract Infection
WFP	World Food Program
WHO	World Health Organization
WV	World Vision

Acknowledgment

I would like to thank and acknowledge the Disaster Risk Management and Food Security Sector [DRMFSS] for the coordination and support during the field work. Special thank goes to Mrs. Almaz Girmay the East and West Harerege Zones Coordinator for her guidance advice and coordination during the assessment. I would like to also thank the Zonal and District Administrators and the respective sectoral offices of Health, Water and Zonal and District Food Security and disaster prevention and preparedness for the information, facilitation and coordination during the data collection, discussion and in-depth interview to carry out the multi – sectoral needs assessment.

Last but not least, I would like to also appreciate the Federal Ministry of Health/EHNRI, Federal – DRMFSS, Oromia Regional – DRMFSS, World Health Organization [WHO] and UNICEF for the coordination, the guidance and the invaluable presentation and orientation during our departure for the assessment.

Kind Regards with Many Thanks

Negusu Worku GEBRMICHAEL
July, 2010

Summery/Abstract

Ethiopia is one of the developing countries where malnutrition, Shortage of water supply, natural unfavorable rain distribution, communicable disease outbreaks and low coverage of latrine - causes a significant contribution for the majority of morbidity and mortality.

In the developing world, malnutrition is one of the leading causes of mortality and morbidity. It is widespread and endemic in all developing countries and may reach *epidemic proportions* as a consequence of natural and/or man-made disasters.

More than one billion people in the developing world have no safe drinking water, or water for washing their food, hands and utensils before eating and 2.4 billion also have no adequate sanitation. This leads to:

- water-borne diseases (e.g. AWD, typhoid)
- water –related diseases (e.g. malaria, yellow fever, river blindness, sleeping sickness)
- water-based diseases (e.g. guinea worm and bilharzia) and
- water-scarce diseases (trachoma and scabies)

All type of diarrhea is a leading killer of children in sub-Saharan Africa. The crisis is worst where 2 in 5 people lack safe water. A baby in Africa is almost 520 times more likely to die from diarrhoea than one baby in Europe.

Improving access to clean water and sanitation would dramatically reduce illness and death in poor countries: a clean water supply reduces diarrhoea-related death by up to 25%, while improved sanitation reduces it by 32%.

In Africa, there are 500 million cases of malaria each year, and up to 1.5 million deaths. The majority in sub-Saharan Africa, which is home to an estimated 80% of the world's malaria cases.

In Africa, children under five years and pregnant women bear the brunt of the disease because of lower or undeveloped immunity. It is the leading cause of death for under-fives in Africa – killing 1.1 million every year. Children that survive may suffer anemia and cerebral problems that affect long-term development.

In East Africa, an estimated 30% of all recorded deaths during pregnancy are attributed to malaria infection. Meanwhile, a recent study in sub-Saharan Africa suggests that another vulnerable group of people – those with HIV – are twice as likely to catch malaria as those without.

Based on the reporting data from various sources related with AWD, the analysis result affirms that, AWD has a seasonal component associated with the rain season. An increase in temperature - increases the number of cases of this sickness within the population. Increasing temperatures aggravate the outbreak and causes the increase of the cholera cases during an epidemic. The climate change is affecting the dynamic and resurgence of infectious sicknesses in a key fashion, concretely malaria and cholera. The outbreak of Cholera highly associated with water scarcity/contamination and poor sanitation due to low number of pit latrines with latrine coverage and utilization.

Methodology

Data Collection was conducted both qualitatively and quantitative using a structured questionnaire for the selected districts for this *Belg seasonal assessment*.

An Indebt interview for the current situation had been conducted at the Zonal and district level for the respective government officials and sector offices including: Health, Agriculture, Water and rural agricultural development.

Selection Criteria for the Visited Districts

The East and West Harerege Zone Multi – Sectoral team visited the selected fourteen districts. The Selection of the districts were based on the Emergency record in the last three years on Health, Food and WASH (Water and Sanitation). Additional districts were visited based on the request from the regional and zonal officials mainly because of the fact that, the current heavy rain damaged *Belg crops and water schemes in some districts*.

General Objective

- To assess the general disease outbreak on Health, Nutrition and Water for the last 6-12 months in the zone and to identify the gap,

Specific Objective

- To conduct a *Belg seasonal* situations in the affected districts and as a result to monitoring and closely observe the impact on Health, Water, and Food security in the through out the two East and West Harerege Zones.

Introduction/Situational Analysis

East Harerege zone is one of the areas in Ethiopia which is at risk of recurrent drought and environmental degradation. Rural Households are vulnerable to crop failure, productive assets depletion and inadequate access of vital resources such as food and water. Negative effects on rural communities include food shortages and widespread severe malnutrition, mainly among women and children.

Unlike the previous years, general food security is beginning to improve as a result of the heavy rain from late April – end of May 2010. The amount and distribution of the rain was fair as reported from the zonal and administrative officials and agricultural and disaster risk management and food security sector experts. But, subsequently the amount of rain was heavy and as a result Belg crops of Potato damaged 80-90% and Maize (20-30%) at the flowering stage.

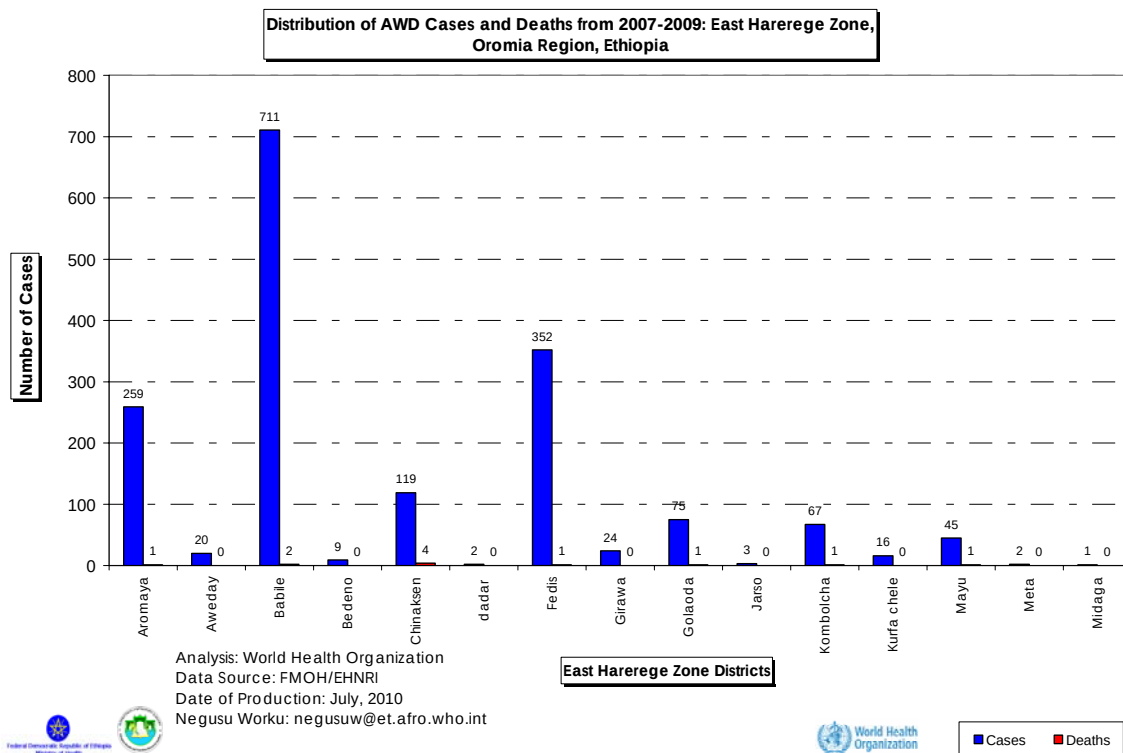
Despite the fact that, the amount and distribution of rain was registered fair at the very beginning, gradually the amount shows declining and completely stopped.

Consequently, there has been potential risk reported from the district health experts that, because of the shortage of food the number of Sever

Acute Malnourished (SAM) cases and the number of Stabilization Center [SC] increasing in the respective health centers and health posts.

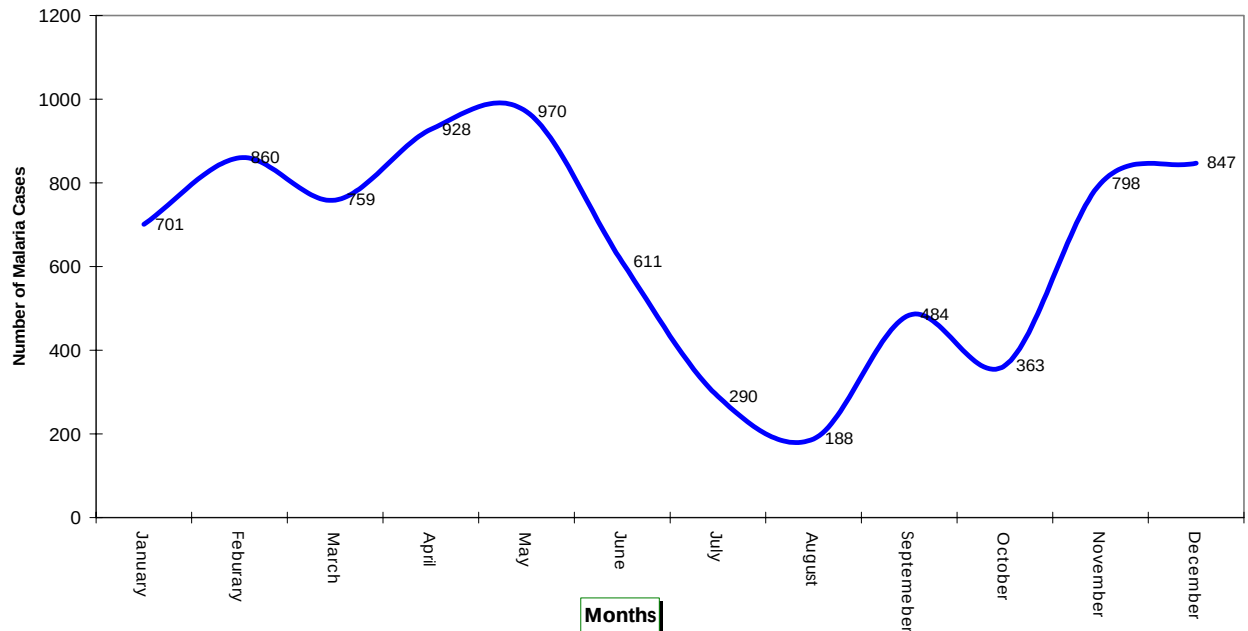
Moreover, Malaria and communicable diseases like Acute Watery Diarrhea (AWD) have potentially emerged in the center of the zone and AWD cases had been increasing from the neighboring “nucleus region” of Harar.

In some kebeles in Kurfa Chelle districts, among the total enrolled students above 50% of the students has been migrating with their family in search of water and the shortage of water and food also affected the teaching learning process at the school.



In East Harerege Zone, despite the AWD outbreak has been controlled for the last 6 months in all woredas, still there is a potential fear of AWD outbreak in the zone because of the high risk places and the neighboring regions and zones reporting active AWD cases from Harari region in Harar town and the West Harereg Zone from eight woredas namely: Me-esso, Gemechils, Anchare, Chiro-Zuria, Guba-Koricha, Dobba, Hawi-Gudina and Daro-labu. By this year from East Harerege Zone, there were 62 cases and 2 deaths with the Case Fatality Rate (CFR) of 3.2 from Goro-Gutu (4 cases and 0 Deaths) and Meta (58 Cases and 2 Deaths) respectively.

In the case of Malaria, 7799 cases were reported from the Zone. Despite, Insecticide Treated Nets (ITNs) coverage shows 85-90% by for the last 3-4 years, replacement process has not been done.

Confirmed All Type of Malaria Cases: East Harerege Zone, Oromia Region, Ethiopia: as of July, 2010

Analysis: World Health Organization
 Ethiopia Country Office
 Data source: East Harerege Zonal Health Bureau
 Date of Production: July, 2010
 Contact: negusuw@et.afro.who.int

**General Background of East Harerege Zone**

East Harerege Zone is located in the Eastern part of Ethiopia. The Zone is a typical example of the country as a whole in terms of vulnerability. Most of the population lives on degraded land with unreliable rainfall. The Zone is made up of 18 administrative districts with a total area of 22, 622.6km². The total population of the zone is 2.980862 and the population density is 131.7/km².

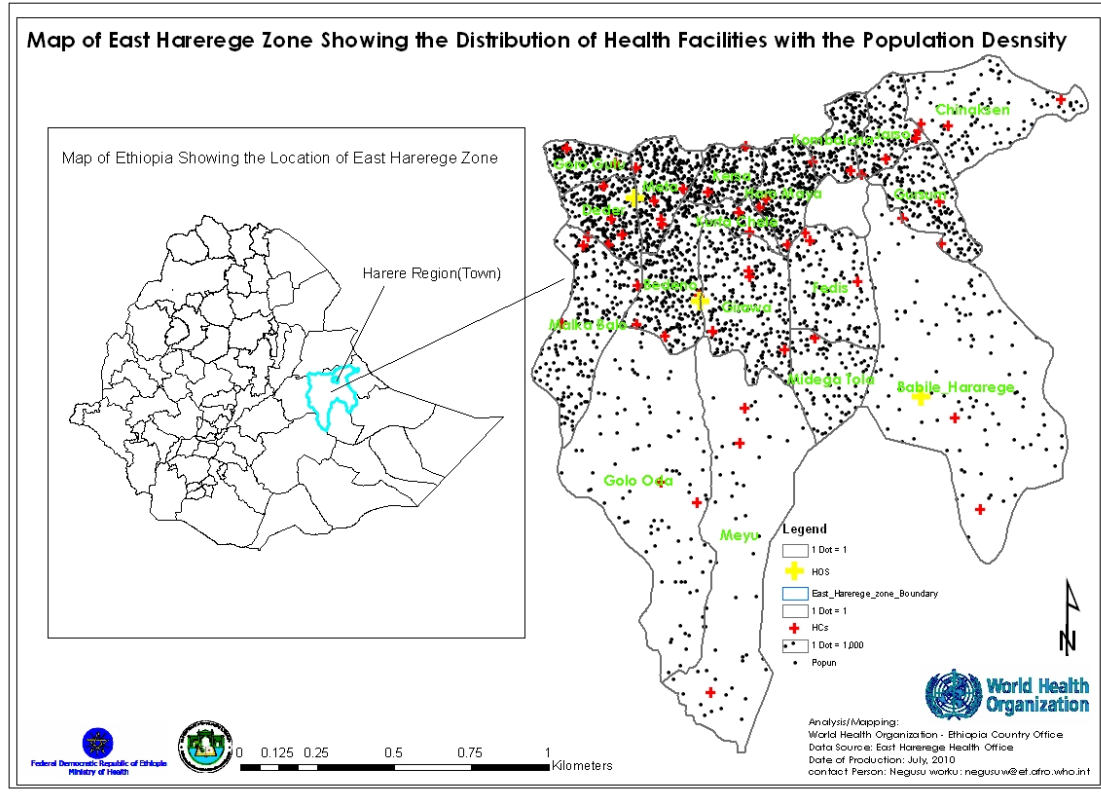


Fig 1: Map of East Harerege Zone showing the Distribution of Health Facilities and with Population density.

Geographically, the zone borders two administrative zones of Oromia region (Bale from the South west and West Harerege from the West), two regional states (Harar (as a nucleus) and Somali in the East) and one administrative council (Dire Dawa from the North).

Agro-Ecologically, East Harerege Zone has four climatic zones: namely: Semi-arid, Steppe, Tropical climates and Tropical Highlands (based on 'Koppen' climatic classifications. The zone's annual precipitation ranges between 700 - 1200mm per annum.

Out of the total 2.9 million populations, 93.96% of them are living in the rural areas and depend for their subsistence on agriculture and related activities. The remaining 6.04 live in small towns.

Health

Primary Health Care [PHC] Coverage in the East Harerege Zone achieved 70.4%. It has a total of 3 Hospitals, 54 Major Health Centers, and 401 health post. The Zone has a total of 2517 health professionals. The ratio of one Medical Doctor and Nurses to the zone is 270,987 and 3536 respectively.

Table 1. Shows the Distribution of Human Resources in the East

Harerege Zone

Table 1: Shows the Distribution of Human Resource on Health in East Harerege Zone of Oromia Region, Ethiopia, July: 2010				
SerNo	Health Professionals	Male	Female	Total
1	Surgeon	1	0	1
2	Medical Doctor (GP)	8	4	12
3	Health Office	62	4	66
4	Pharmacist	7	2	9
5	Nurse BSc	57	13	70
6	Physiotherapist	2	1	3
7	Psychiatry Nurse	0	0	0
8	Pediatric Nurse	0	0	0
9	Ansth. Nurse	1	0	1
10	Dental Nurse	2	1	3
11	BSc Dental Nurse	1	1	2
12	OR Nurse	0	1	1
13	Ophthalmic Nurse	0	0	0
14	Senior Clinical Nurse	216	138	354
15	Junior Clinical Nurse	58	62	120
16	Senior Public Health Nurse	33	6	39
17	Junior Public Health Nurse	14	2	16
18	BSc-Midwife Nurse	2	1	3
19	Senior Midwife Nurse	14	15	29
20	Junior Midwife Nurse	2	4	6
21	Other Nurses	5	7	12
22	Lab technologist BSc	23	3	26
23	BSc Environmental Nurse	28	0	28
24	Senior Environmental Nurse	40	0	40
25	Health Assistant	29	5	34
26	Senior Lab Technician	34	9	43
27	Junior LAB Technician	0	1	1
28	Druggist	58	15	73
29	Junior Pharmacy Technician	1	3	4
30	Senior X-Ray Technician	1	0	1
31	Junior X-Ray Technician	1	0	1
32	Health Extension Worker	0	1041	1041
33	Others	5	0	5
34	Admin. Assistant	220	324	544
Grand Total		925	1663	2588

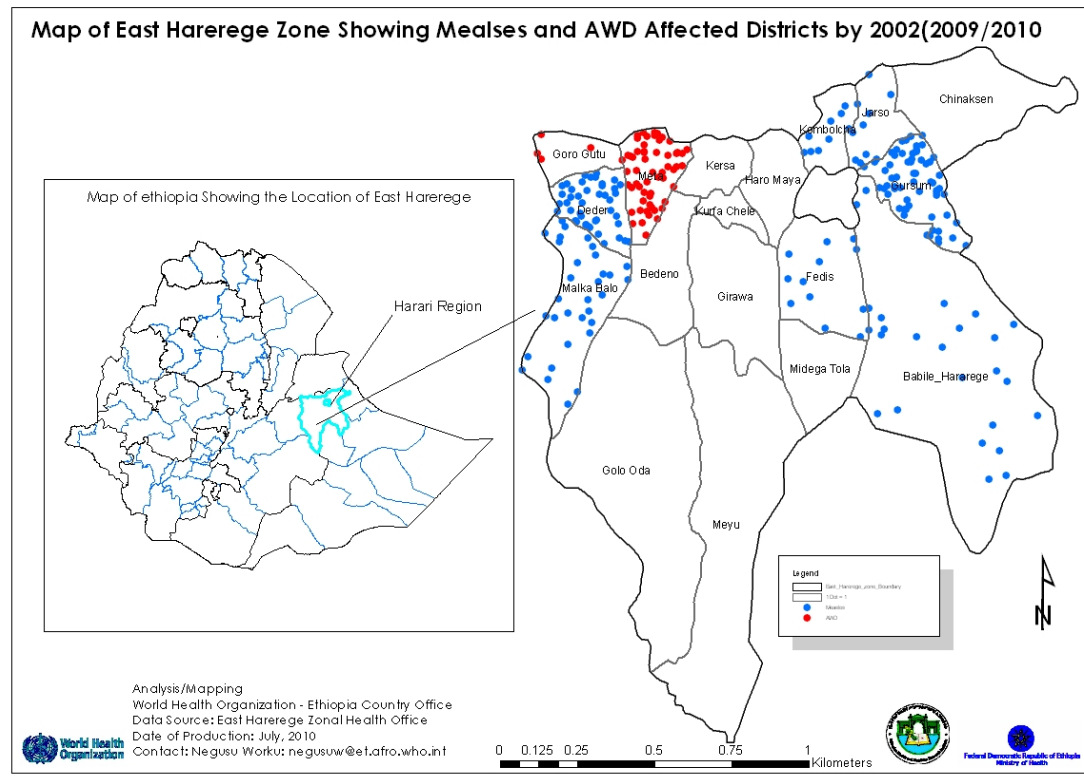
Source of Data: East Harerege Zonal Health Department

During 2002 in Ethiopian Calendar (2009/2010), a total of 62 AWD Cases and 2 Deaths with CFR of 3.2 reported from two districts of Goro-Gutu (4 Cases) and Meta (58 cases with 2 Deaths).

A Total of 195 Measles Cases and 1 Deaths were also reported from seven districts of in East Harerege Zone with CFR of 0.5.

Table 2. Shows Measles Epidemic Cases by the Year 2002 (2009/2010)

Distribution of Measles Cases and Deaths as of 2010						
SerNo	Districts	Popn	Cases	Deaths	CFR	AR
1	Babile	102552	24	1	4.2	23
2	Deder	249104	45	0	0.0	18
3	Fedis	124222	13	0	0.0	10
4	Gursum	168474	63	0	0.0	37
5	Jarso	129594	8	0	0.0	6
6	Kombolcha	153254	10	0	0.0	7
7	Melkabelo	192097	32	0	0.0	17
Total		1,119,297	195	1	0.5	17



Ten Top Diseases in East Harerege Zone

1. Fighting
2. Pneumonia
3. Dysentery of Skin
4. ARI
5. ALL forms of Dysentery and Ameba
6. All forms of Intestinal Parasites,
7. Gastric Intestinal,
8. Other Respiratory Diseases
9. UTI and
10. All forms of Malaria

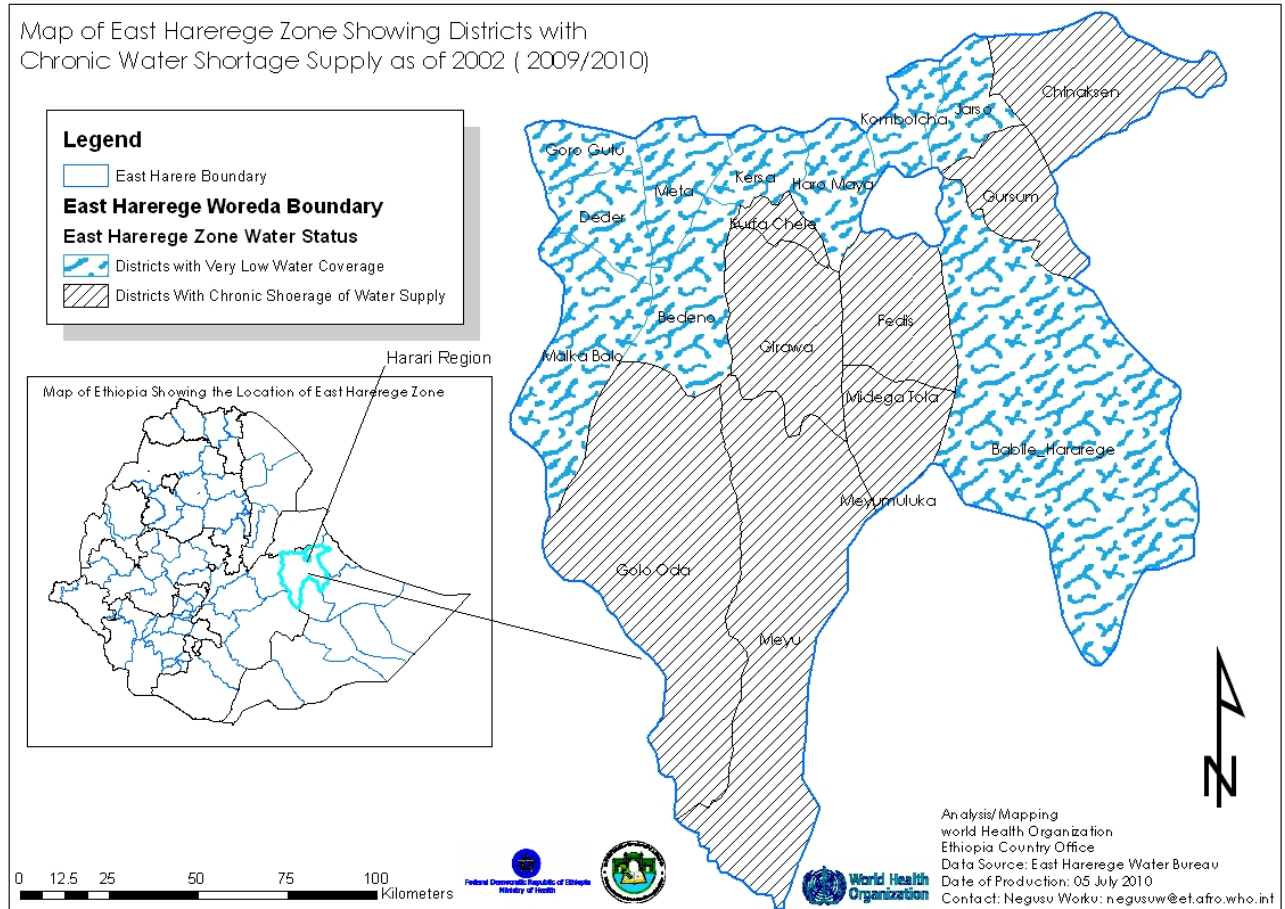
Water and Sanitation

According to the East Harerege water bureau, the water coverage of the zone is 48.65%. The remaining 1.77 million suffering from the shortage of water supply.

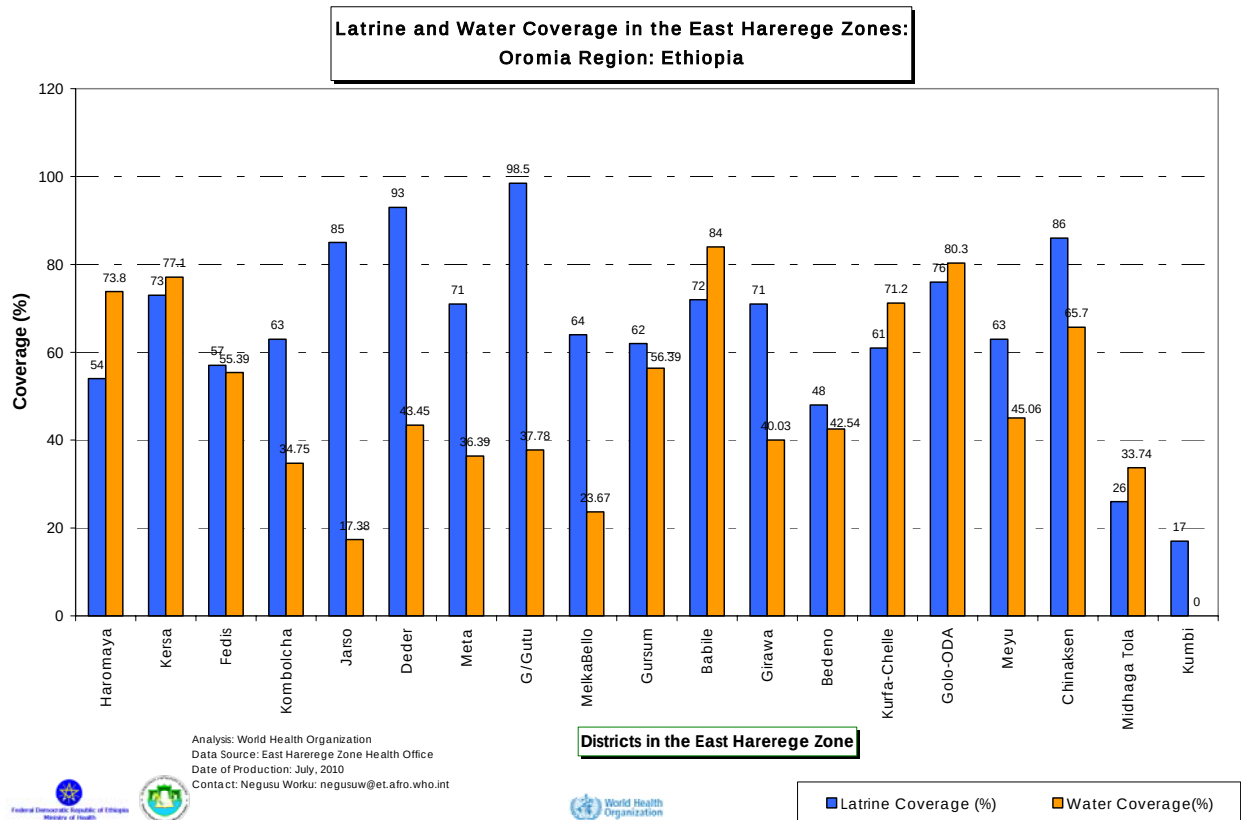
According to the official zonal data indicating that, there are six districts Completely labeled as chronic water shortage including: Midaga, Meyu, Kumbi, Gursum, Chenakse and Fedis.

Table XXX shows the East Harerege Zone Water Coverage

Districts	Total Popn	Popn Supply Potable Water	Coverage (%)
Haromaya	201793	148932	73.8
Deder	203286	88333	43.45
Goro-Gutu	134016	50632	37.78
Metta	219684	79938	36.39
Qarsa	160774	123964	77.1
Badano	222335	94796	42.54
Melka-Bello	179673	41342	23.67
Gola Oda	99492	79888	80.3
Gursum	132194	74548	56.39
Kurfa_Challe	64681	45053	71.2
Jarsso	121172	21054	17.38
Midhaga Tola	70599	23818	33.74
Meyu	53554	89873	45.06
Girawa	224487	59023	40.03
Fedis	106565	54388	55.39
Chinaksen	82775	58412	65.7
Babile	69538	49038	84
Kombolcha	141104	141104	34.75
Kumbi	NO DATA		0
Total	2,487,223	1,208,165	48.65%



Moreover, because of the shortage of water supply and low coverage of Latrine in the zone, there is a high potential of AWD Out break in the zone and the neighboring “nucleus” region of Harari, Somali and Oromia regions via common religious activities and sharing common source of unprotected rivers and ponds in Messo and Somali boundary. Based on the reporting data, all AWD cases were reporting from those districts with no water supply at all with low coverage of latrines. Currently, Harari and Somali regions reporting active AWD cases and there is a high risk of out breaking in the zones.



Based on the above fact data, the zonal latrine and water coverage is 68 and 48.7% respectively which is below coverage. Since it aggravate the outbreak situations in the zone, health education and latrine utilization work has to be done in line with the water coverage. Potential hazards of contaminating water source are high since the zone population density is very significant and the living standard, socio economic status and accessing drinking water are also below standard.

Table XXXX Shows the Distribution of Latrine Coverage in East Harerege Zone

East Harerege Zone Latrine Coverage by Districts

Districts	No-HHs	No of Latrine	Coverage
Haromaya	56311	30658	54
Kersa	39108	28705	73
Fedis	25880	14693	57
Kombolcha	31928	20118	63
Jarso	26999	22846	85
Deder	51896	48493	93
Meta	57103	40753	71
Golo Gutu	32601	32119	99
Melka-Bello	40020	25695	64
Gursum	35098	21889	62
Babile	21365	15316	72
Girawa	54469	38755	71
Bedeno	54124	25779	48
Kurfa Chelle	13323	8152	61
Golo Oda	18046	13781	76
Meyu	10594	6659	63
Chinakesen	20222	17402	86
Midaga	17184	4493	26
Kumbi	6134	1044	17
Haromaya Town	3826	2630	69
Awedaye Town	1758	1998	114
Deder Town	3023	2260	75
Zonal Coverage	621012	424238	68.3

Source: East Harerege Zonal Health Office

Nutrition

Type of Program	Group age	Total Beginning Of The Month/Quarter/	New Admissions		Transfer from another TU	Total Admissions	DISCHARGES			TRANSFER OUT		Total DISCHARGES	Total End Of month/Quarter/
			W/H <70%MUAC<110 cm(children) or MUAC <80mm (adults)	OEDEMA			CURED	DEATH	DEFAULTER	To out-patient	To In -patient		
IN-PATIENT	< 6 month	0	0	0	0	0	0	0	0	0	0	0	0
	6 - 59 month	14	19	38	17	88	21	5	6	33	0	65	23
	5 - 10 Years	0	0	0	0	0	0	0	0	0	0	0	0
	11 - 17 years	0	0	0	0	0	0	0	0	0	0	0	0
	>18 year	1	1	0	0	2	1	1	0	0	0	2	0
	Total	15	20	38	17	90	22	6	6	33	0	67	23
OUT-PATIENT	< 6 month	0	0	0	0	0	0	0	0	0	0	0	0
	6 - 59 month	263	78	36	1	378	98	0	7	0	2	107	271
	5 - 10 Years	0	0	0	0	0	0	0	0	0	0	0	0
	11 - 17 years	0	0	0	0	0	0	0	0	0	0	0	0
	>18 year	0	0	0	0	0	0	0	0	0	0	0	0
	Total	263	78	36	1	378	98	0	7	0	2	107	271



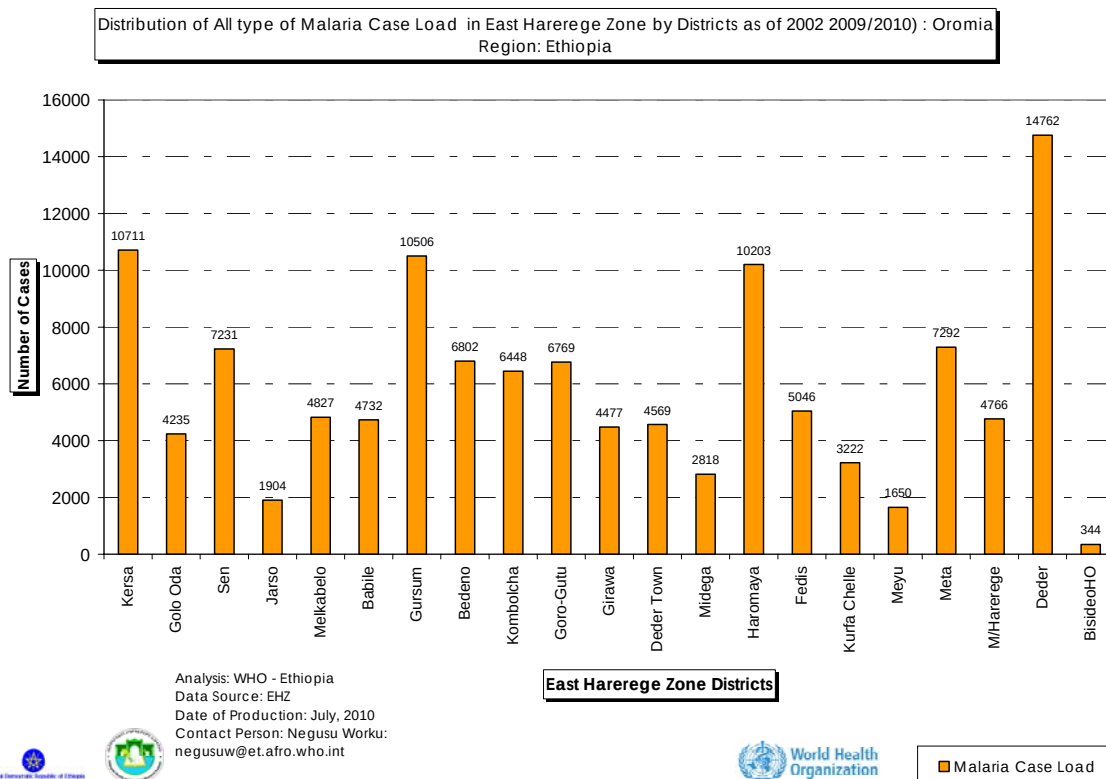
Belg – Multi – Sectoral Humanitarian needs assessment from June 22 – July 12, 2010: By Negusu Worku GEBRMICHAEL [negusuw@et.afro.who.int]

Malaria

	All Cases Treated only Clinically			All Malaria Cases by Age			Cases Treated in the Health Facilities with Laboratory Services								Admitted		Inpatient		Malaria in Pregnancy			Total
Districts	Total	<5 yrs	>5 yrs	Total	<5yrs	>5yrs	Pos	PF	PV	Mix	Male	Female	<5yrs	>5yrs	Total	Malaria	Total	Mal	OutP	InP	Deaths	All Malaria Cases
Kersa	7439	1894	5545	1549	149	1400	427	418	9	0	212	215	21	406	12	2	0	0	0	0	1	10711
Golo Oda	3695	908	2787	540	79	461	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	4235
Sen	5854	1954	3900	1357	109	1248	5	3	2	0	3	2	0	5	0	0	0	0	0	0	0	7231
Jarso	1484	634	850	400	39	361	5	5	0	0	3	2	0	5	0	0	0	0	0	0	0	1904
Melkabelo	4071	1179	2892	438	75	363	79	32	47	0	51	28	20	59	2	0	0	0	0	0	0	4827
Babile	3051	851	2200	572	196	376	223	186	37	0	114	109	19	408	13	0	0	0	0	0	0	4732
Gursum	9904	3183	6721	546	24	522	11	7	4	0	4	7	3	8	12	0	0	0	0	0	0	10506
Bedeno	6363	897	5466	431	46	385	0	0	0	0	0	0	0	0	8	0	0	0	0	0	0	6802
Kombolcha	6272	873	5399	176	24	152	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	6448
Goro-Gutu	5421	878	4543	945	85	860	95	86	9	0	56	39	23	72	21	2	0	0	0	0	0	6769
Girawa	3982	832	3150	487	12	475	2	2	0	0	1	1	0	2	0	0	0	0	0	0	0	4477
Deder	4487	1285	3202	59	2	57	1	1	0	0	1	0	0	1	19	0	0	0	0	0	0	4569
Midrega	2671	1332	1339	147	48	99	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	2818
Haromaya	7041	2228	4813	2618	38	2560	136	135	1	0	72	64	34	102	0	0	0	0	0	0	0	10203
Fedis	4695	1457	3238	270	70	340	15	15	0	0	7	8	1	14	21	0	0	0	0	0	0	5046
Kurfa Chelle	2943	603	2340	255	56	199	6	0	6	0	5	1	0	6	0	0	0	0	0	0	0	3222
Meyu	1631	804	827	19	1	18	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	1650
Meta	6969	1320	5649	253	42	211	16	13	3	0	10	6	3	13	4	0	2	0	0	0	0	7292
M/Harerege	3213	1380	1833	1480	95	1385	12	10	2	0	6	6	3	9	18	1	0	0	3	3	0	4766
Deder Town	13960	2154	11806	67	1	67	0	0	0	0	0	0	0	0	735	0	0	0	0	0	0	14762
BisideoHO	0	0		77	7	70	0	0	0	0	0	0	0	0	267	0	0	0	0	0	0	344

Based on the above official data from the East Harerege Zone depicts that, the entire district of East Harerege zone has been affected by the all type of Malaria cases by the year 2002 (2009/2010). All of the districts have been labeled as Malarious woreads.

Despite the fact that, ITNs Coverage was 80-90% for the last three years, the old net became out of use and needs replacement and a high demand request has been delivered both to the districts and the zonal malaria control department.



Result East Harerege Zone

Kurfa-Chelle District

Kurfa-Chelle: district located in Northern part of the East Harerege Zone. It has a total population of Despite the district obtained a very good rain in early January by amount and distribution, the Belg season Potato crops had been total damaged because of the excess water and flood. The Potato production contributes one third of the daily staple food.

Consequently the number of OTP sites became now 18 and there are also two Stabilization Center (SC).

Additional food and drug supply has been mobilized from CARE and UNICEF international organizations.

In the district there are 4 Peasant Associations (PA) that has been severely affected by shortage of water supply by *Belg Season including*:

- 1. AFERKELLO,**
- 2. DIRE-GUDINA,**
- 3. IJE KECHER AND**
- 4. ORDE.**

As a result of the shortage of water supply: students have been migrating for in search of water and food. A teaching and learning process has been highly affected.

AWD out break and other diseases has been reporting from the district a month before. There is a high threat to be affected again since there have been AWD cases actively reporting from neighboring districts and from Harar town.

In the district there are two health centers and one under construction.

A total of 39 Health Extension Works have been attached in every PA's for assessing and conducting health education for the rural community.

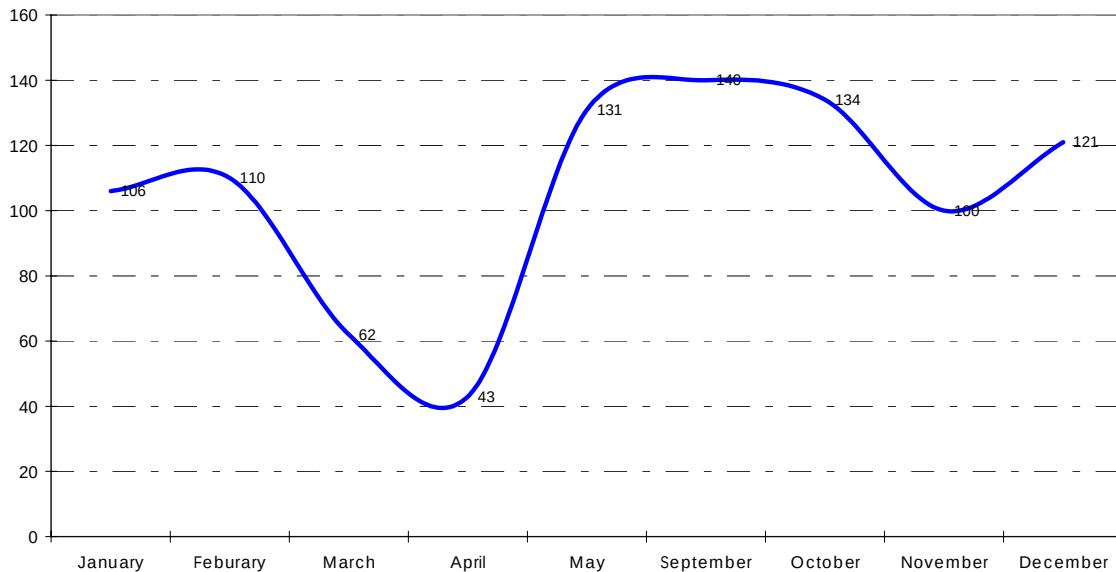
In the districts the top five major morbidity and mortality diseases are:

Pneumonia, Diarrhea, Anemia, Intestinal Parasites and accident.

From September 2009 - May 2010 (Meskerem – May 2002), there was no cases and outbreak of AWD, Meningitis and Measles.

In the Case of Malaria 947 Cases of all type of Malaria cases were reported.

Trend of Seasonal Malaria Cases, Kurfa-Chelle District, 2010



Analysis: WHO - Ethiopia
 Data Source: Kurfa-Chelle District Health Office
 Date of Production: July, 2010
 Contact: Negusu Worku: negusuw@et.afro.who.int



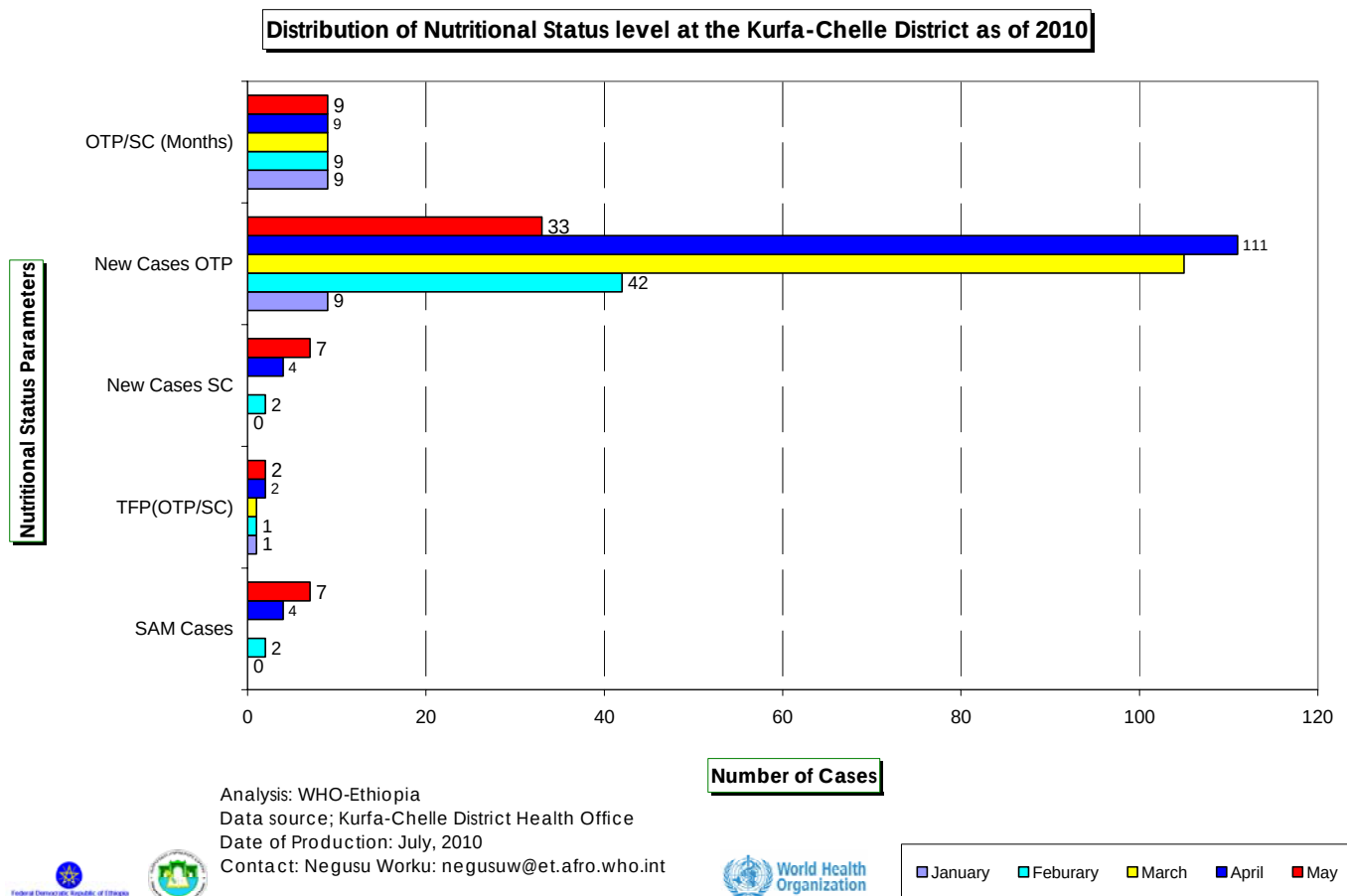
— Malaria Case Load

Moreover, Measles Vaccination coverage u-1, Pentavalent Vaccination u-1 and ITNs coverage in the district is 58%, 54% and 90 respectively. Despite the fact that the ITNS coverage is fine, there has a compliant report from the district that, a replacement demand has been increased from day to day both from the community and the district to the zonal health office.

Since the district has fulfill all the Malrious parameters of altitude, and endemic the replacement process has to be begin now and immediately. This issue has been repeatedly stressed during the debriefing report to

the zonal administrative officials, the zonal health office officials and the Oromia Regional Health Bureau (ORHB) experts and representatives.

Nutritional Status at Kurfa-Chelle District



Girawa District

Girawa district is located in the central part of East Harerege Zone. It has a total population of 268,640. Among them under 5 population contributes 17% which is nearly 44, 100.

The district has further divided into lowest administrative level called Kebeles. There are 45 rural Kebeles and 1 urban administrator.

The District has a total of 10 Health Centers, 45 Health Post. The Primary Health Coverage in the district is nearly 90%. This year there hasn't been outbreak of AWD, Malaria, Measles and Meningitis.

Despite the water coverage in the district is only 18.5%, percentage of district population using improved water source and percentage of district population getting water from treated source is 40%. Among the total 47 Peasant Association, 16 of them or nearly 35% of the Kebeles has been suffered by **Chronic Water problem.**

Latrine coverage and utilization in the district by the year 2009 is 84% and 54 respectively.

In the district, there are nearly 90 health extension workers. AWD preparedness work has been done since there is active cases has been reporting from the neighboring district.

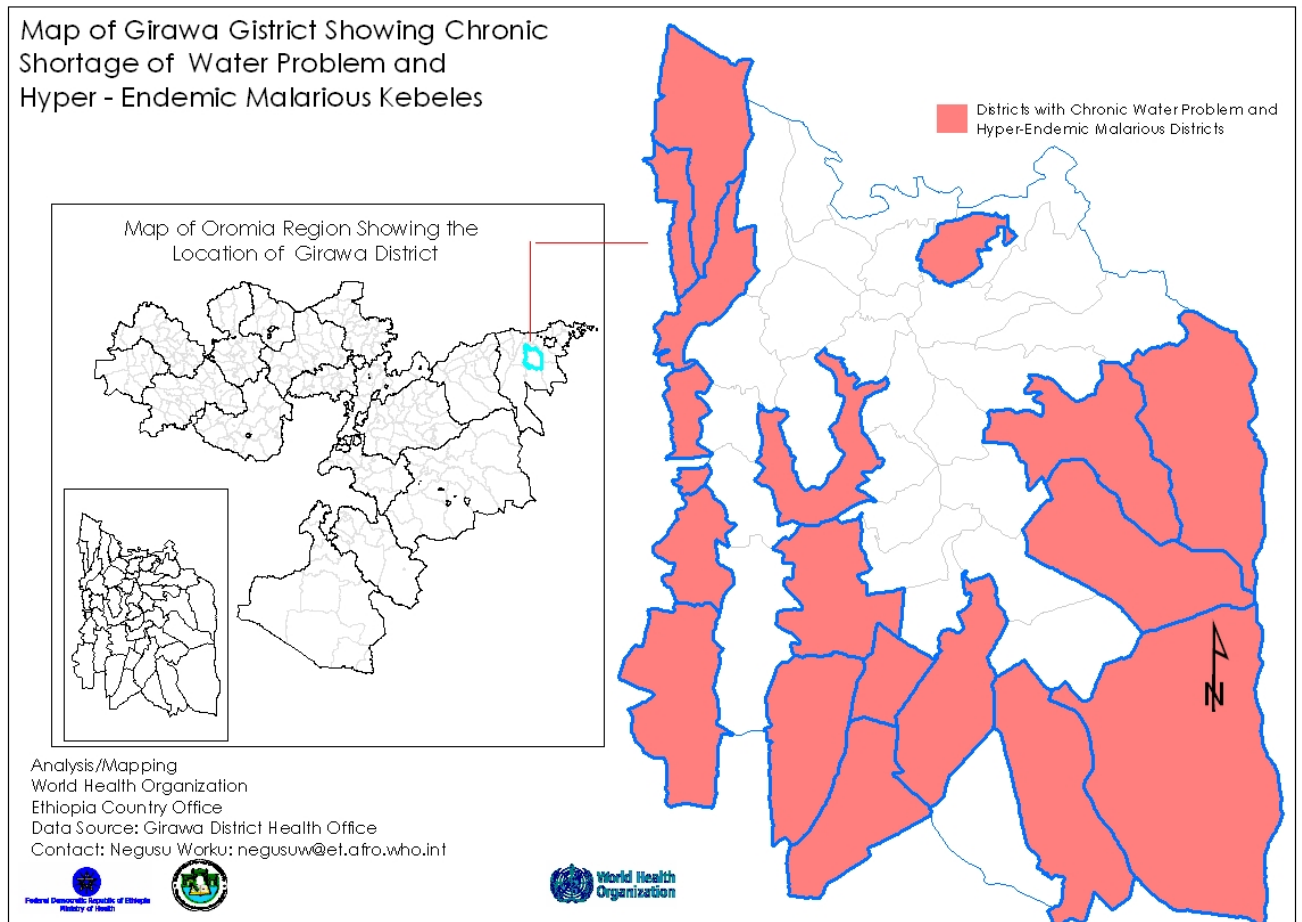
There are risk factors for the outbreak of Malaria in the district since half of the kebeles has been identified as hyper-endemic Malarious area.

Recent data from the district shows that, ITNS replacement (Coverage) in the second round after 3-4 years is Zero. This is a very shocking news since the districts has all the parameters to the outbreak of malaria.

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including: *altitude <2000, there is malaria endemic area, there is also sites of malaria breeding, potentials interrupting rivers and unprotected irrigations are there.*

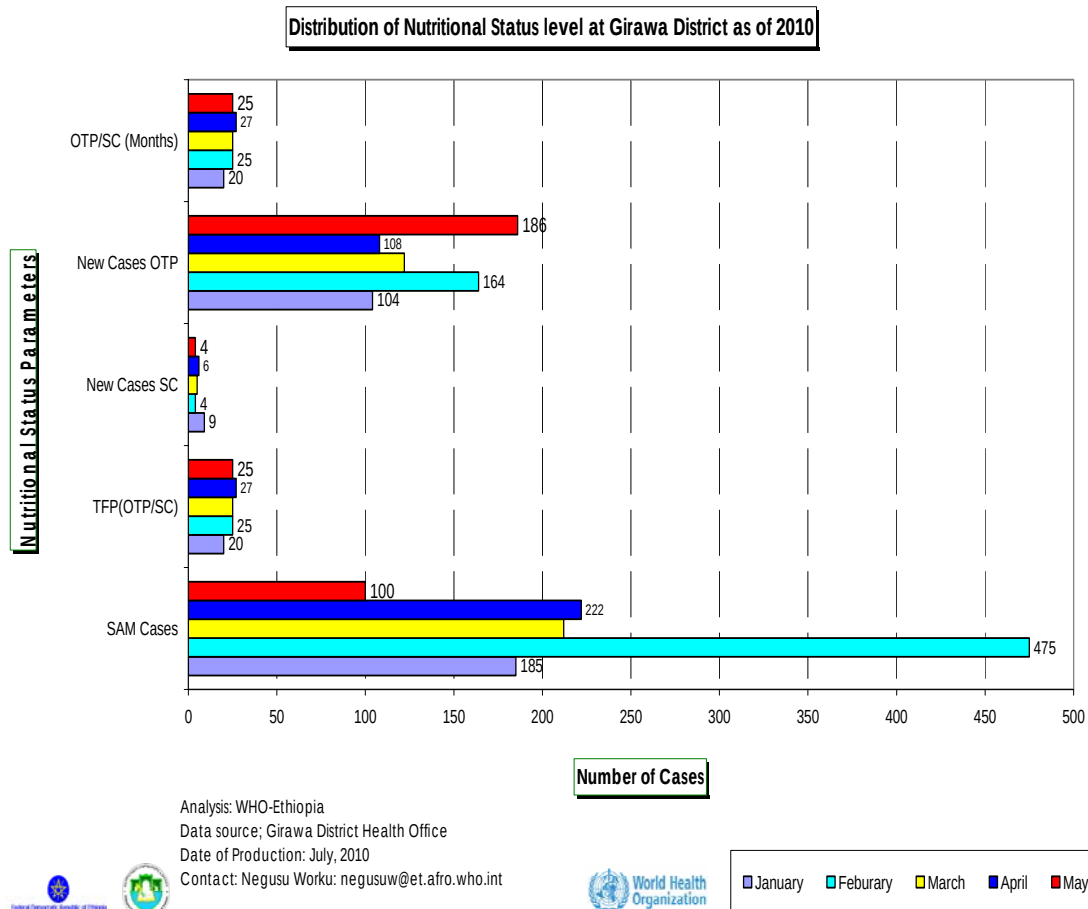
In the district, low-land areas obtained rains. Consequently, there is a fear of Malaria outbreak. This is mainly because of the fact that the ITNS replacement coverage in the district is zero.



Nutrition Status in Girawa District

In every six months interval, there is a nutrition screening process. In this *Belg* season as reported from the district officials - there is a problem of shortage of food supply both in the Highland and Low-land areas.

As a result 36 OTP sites were opened. Additional food and food related items have been mobilized via IRC and CARE international organization. Therapeutic Supplies were enough.



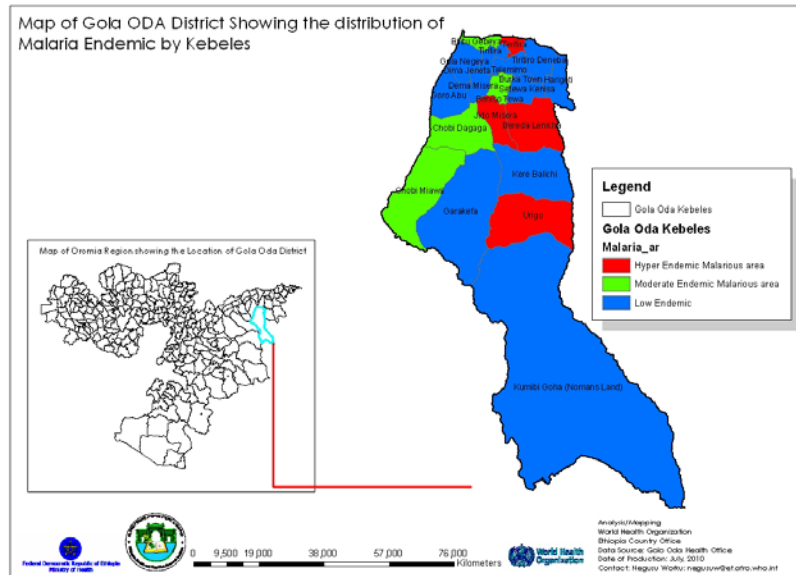
Gola ODA District

Golo Oda district is located in the South Western part of the East Harerege Zone. It has a total population of 86,620 as of the new census report. The district further divided in to the lowest administrative levels called Kebeles. The District has a total of 18 Kebeles.

There are three Health Centers, 17 Health Post, 29 Health Extension Worker and a total of 13 Registered Nurses (RN).

Among the total 18 Kebeles, 4 of them have been marked as Hyper-endemic Malarious Kebeles and a serious of chronic shortage of water supply namely: **Bereda, Jedo, Majo and Tirtere Deneba** , 5 of them Moderate and the remaining has been labeled as Low Malarious endemic Kebeles. (Please see the map below).

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Water coverage in the Golo Oda district is 38%. All of the kebeles have a chronic shortage of water supply. Among them there are five PAs which has a serious of chronic water supply and high sulfur chemical problem in the Golo Nega PA including:

1. *Golo Nega,*
2. *Majio*
3. *Terterio Deneba,*
4. *Jido and*
5. *Berede*

The District latrine coverage and utilization coverage is 60% and 20%.

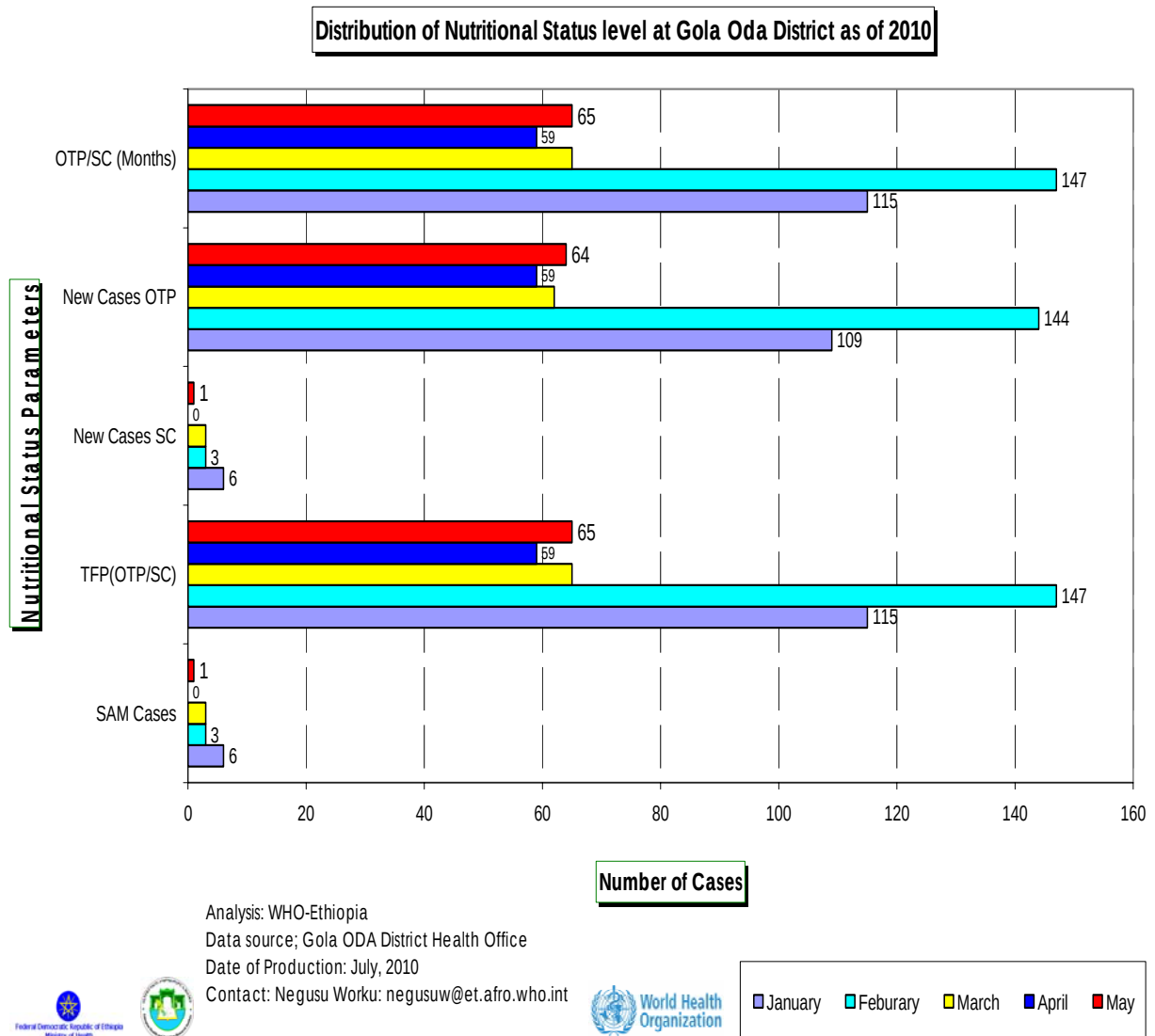
The WASH program in the district has been supported by the World Bank.

Despite the fact that AWD has been controlled for the last three years, preparedness activities have not been done. Mainly because of the fact that, there is budget allocated from the zone as well as from the donors.

ITNs coverage in the district for the last three years was 80% and replacement has to be done – since the district is Malaria endemic.

Nutrition Status in Gola ODA District

There are 12 OTP sites and one SC in the district. Most case load of OTP has been decreed as compare to the previous year. The district has been accessing nutritional components from the Zonal Health office.



Bedeno

Bedeno district is located in the North Western part of East Harerege Zone. It has a total population of 295,797. The district has 40 rural Kebeles and of which 18 of them are Malarious hyper-endemic, 6 labeled as Moderate and the remaining 16 are non-malarious area. Water, Latrine and Latrine utilization coverage in the district is 42%, 46.3% and 38.9%. There are five Kebeles in Bedeno district with a complete shortage of water supply including:

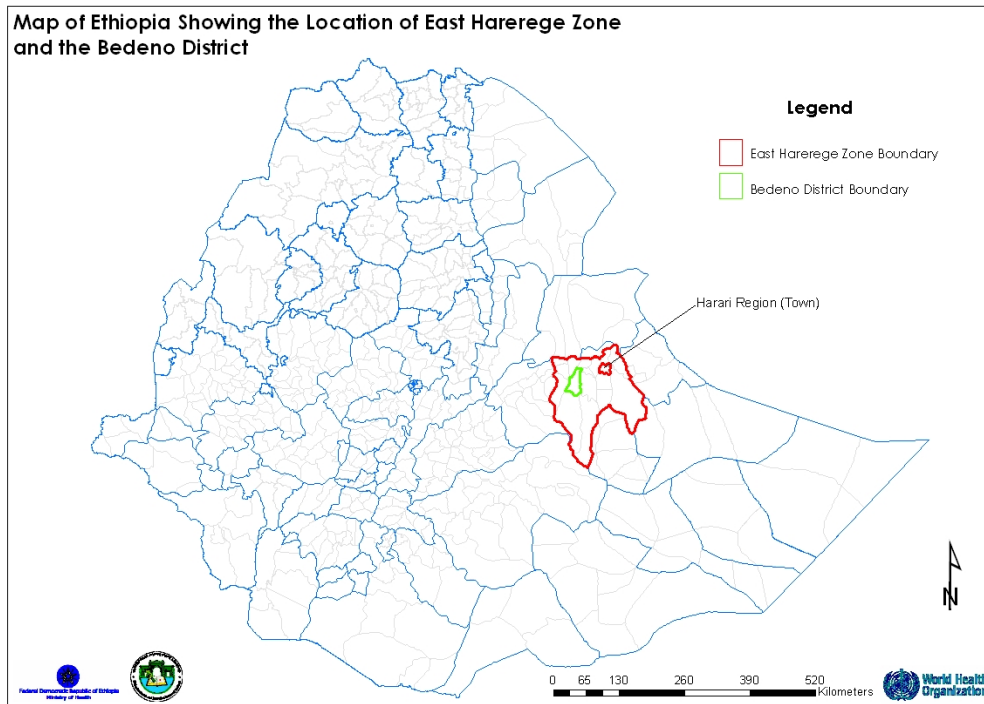
- 1. Bortole,**
- 2. Oda Beshani,**
- 3. Giro Belina,**
- 4. Tortora Guda and**
- 5. Harro Kersa**

The District has a total of 3 Health Center and 23 Health Post.

The Major top five morbidity and mortality cases are **Soft tissue**

Problem, Intestinal Parasites, Gastric, Pneumonia, UTI and

Malnutrition Problem.



Despite there is no as such unusual outbreak has been occurred, shortage of food problem has been observed in the Highland areas mainly because of the heavy rain impact on the *Belg season crops*.

Currently at the district, 300 children sent to 18 OTP sites and 2 Stabilization Center. Supplementary food has been provided by the CARE international and Mother/Child health care services have been given by the IFHP.

Community Based Nutrition Program (CBNP) Iodine Encapsulations has been given two times a year. Half of the Kebeles within the district exposed to Malaria and water born diseases. Despite ITNs has been given 3-4 years before - replacement has not been done. The demand is very high and the zonal and district health officials and administrators reported the issue repeatedly to the Oromia regional health bureau and to the donors.

Trend of OTP cases in Bedeno District

Month	Total new admission cases in all OTPs	Total new admission cases in high land OTPs	Remark	% share of high land cases from total
Dec-09	131	40		31
Jan-10	86	27		31
Feb-10	118	46		39
Mar-10	119	44		37
Apr-10	126	50		40
May-10	96	45		47
Jun-10	211	103	In this month the no of oedematus cases show slight increment	49

Gursum District

Gursum district is located in the North Eastern part of the East Harerege Zone. It has a total population of 154,600. The District has 39 peasant associations and 1 administrator town. Water Coverage in the district is 34.4% and the remaining 51 % of the Kebeles has always been suffering by the shortage of water supply. Additional five Kebeles has a chronic water problem and has no underground sources including:

- 1. Bekeka,**
- 2. Kuleme**
- 3. Warataye,**
- 4. Were-Guye, and**
- 5. Nuleye**

Because of the shortage of food in the district, 375 malnourished children were admitted in 20 OTP sites and in **three Stabilization Center**. Additional Measles outbreak has been also emerged in the district and affected **Nine** PA's including: **Hariro, Misra, Belisuma, Haro-Bati, Abadir, Bekeka, Kuleme, Nuleye, Waraleye, and Were-Guye**.

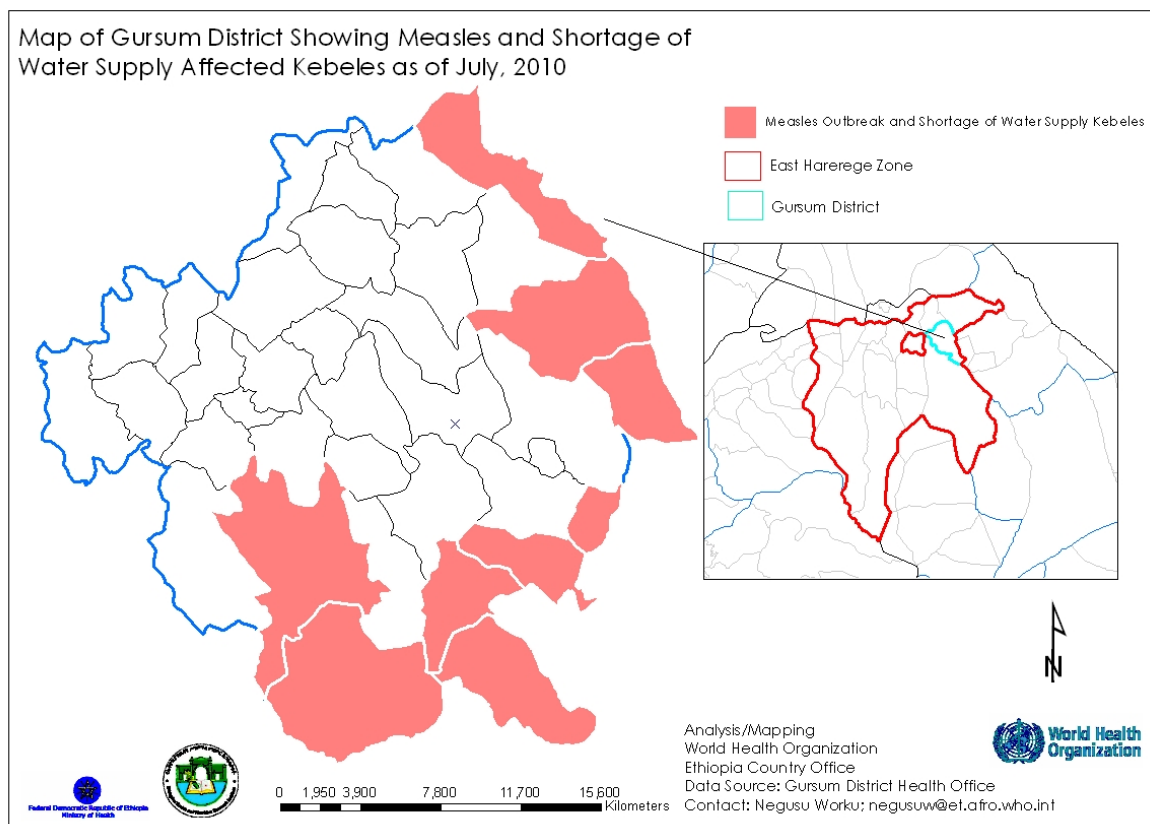
Currently Measles outbreaks on going in the district with a total of 61 confirme cases were reported. Despite the last AWD cases were reported in 2006/2007 - Emergency Preparedness committee in the district had been established. Because there is active AWD cases are reporting from the neighboring towns of Harar. Some cases of bloody diarrhea cases have been also seen in the health facilities.

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In the Gursum District, among the total peasant association, 11 PAs are hyper endemic kebeles including: **Audeye, Oda-Oromia, Nureselame, Oda-Setale, Misra, Bilusima and Hariro.**

ITNs coverage in the district is 70% and replacement request has been done for 19 PAs and a total of 28,000 ITNs were given.

Moreover, all the 11 Malaria-hyper-endemic districts were also affected by shortage of food, Water problem and malnourished. A serious attention has to be given to these PAs since the magnitude of the problem has been complicated and significant as compare to the other PAs within the district. [Pls See the Map below]



Summery of East Harerege Zone	Visited Districts in the East Harerege Zone during the Assessment				
Major Indicator Variables	Girawa	Kurfa-Chelle	Gola-Oda	Bedeno	Gursum
Measles Vaccination in u-1%	83%	58	NA	91	81
Pentavalent Vaccination in u-1%	116%	54	NA	100+	95
ITNs Coverage%	80*	90*	84*	85*	70*
Woreda IDSR timeliness rate L6months%	90	50	75	50	80
% of District Popn using improved Water	20	40	38	19	34
% of District Popn getting water from treated sources	20	40	36	19	34
Latrine Coverage%	71	83	60	46.3	51
Latrine Utilization%	49	54	20	38.9	43
Health Partners in the Woreda	CARE, IRC	CARE	IMC	CARE, IFHP	IMC, CARE
Population	261,452	63,950	86, 620	259,797	168474
U5	44,057	10232	14206	44,166	26956
Above 5	224,583	53718	72414	215,631	138518
Water Coverage	40.3	71.2	38	42.54	34.4
Free Health care Services	TBL, Vaccination, ART, FP, Maternal and Child Health	Maternal Services, EPI, FP, Nutrition	TBL, EPI, Malaria, Malnutrition	Malaria Rx, TBL, MCH, Malnutrition	Maternal Services, EPI, FP, Nutrition
Waiver System Covers	All Curative Services	All Curative Services	All Curative Services	All Curative Services	All Curative Services
Top 5 Diseases	PNEMONIA, DIARREHA, ANEMIA, IP, MALARIA	PNEMONIA, DIARREHA, ANEMIA, IP, ACCIDENT	INJURY, PNEMONIA, IP, MALARIA, UTI	Soft tissue injury, IP, GASTRICS, PNEMONIA	PNEMONIA, DIARREHA, GASTRICS, MALARIA
Nutrition					
Total SAM Cases as of 2010	1269	13	13	103	948
Total no of TFP(OTP/SC)	32	9	451	28	11
Number of New Cases SC	32	13	13	2	3
Number of New cases OTP	1269	300	438	211	375

Emergency Health and Nutrition Needs Assessment Requirement					
Intervention	Three Months	Three Months	Three Months	Three Months	Three Months
Activities	Supplementary food	Supplementary food	Supplementary food	Supplementary food	Supplementary food
Total estimated Beneficiaries		10488 People			
Quantities		874 Quintal			
* ITNs had been distributed some four years back. It has been requested to be replaced by the new one.					

Summery of East Harerege Zone	Visited Districts in the East Harerege Zone during the Assessment			
Major Indicator Variables	Meta	MelkaBello	Midaga	Fedis
Measles Vaccination in u-1%	na	94	63	46
Pentavalent Vaccination in u-1%	na	104	78	65
ITNs Coverage%	80*	10*	0*	0 (No Replacement)
Woreda IDSR timeliness rate L6months%	60	80	96	na
% of District Popn using improved Water	26	53	0	34.01
% of District Popn getting water from treated sources	10	na	0	na
Latrine Coverage%	79	72	na	34
Latrine Utilization%	74	60	na	79
Health Partners in the Woreda	GOAL, ESHE	GOAL	IMC	IMC
Population	274,098	182,605	82,482	124,222
U5	45,226	29217	13197	19876
Above 5	228873	153,388	69,285	104,346
Water Coverage	36.4	23.7		
Free Health care Services	ALL Medical Services	ALL Medical Services	ALL Medical Services	ALL Medical Services
Waiver System Covers	ALL Curative Services	ALL Curative Services	ALL Curative Services	ALL Curative Services
Top 5 Diseases	LRTI, Injury, IP, Diarrhea, UTI	Pneumonia, IP, diarrhea, Injury, GI	Malnutrition, ARI, Injury, IP, Malaria	na



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Nutrition				
Total SAM Cases as of 2010	1405	401	281	1276
Total no of TFP(OTP/SC)	37	115	90	95
Number of New Cases SC	39	61	na	44
Number of New cases OTP	583	340	281	432
Total no of OTP/SC for this Month (Jan--May, 2010)	35	95	47	60

* ITN Replacement has been Started

Summery of East Harerege Zonal Level Information

East Harerege Zonal Level Data		
SerNo	Variables	Parameters
1	Is there a functional multi-sectoral coordination forum for the health sector	Yes
2	Are all relevant government, NGOs, and UN agencies represented?	Yes
3	Frequency of Regular Meeting	Yes
4	Does the zone developed multi disease EPRP?	Yes
6	Is there a PHE Preparedness fund?	No
7	If Yes, specify the amount	NA
8	Does the RHB have accessible PHE response fund?	No
9	If yes, specify the fund	NA
10	Is there a dedicated PHEM unit	Yes
11	Number of staffs in the team?	2
12	PHEM team members trained on	
13	PHEM concept and operational framework	Yes
14	IDSR	Yes
15	EPRP	Yes



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16	Outbreak Management	Yes
17	Is there in the Stock	
18	Meningitis Vaccination	N
19	Drugs	
20	Coartem	Yes
21	Oily CAF	No
22	Doxycycline	Yes
23	Ringer Lactate	Yes
24	ORS	Yes
25	LAB Supplies	
26	RDT	No
27	Pastorex (Meningitis)	No
28	CTC (AWD)	No
29	Major Health Challenges	1. Lack of Budget for Emergency Activities like training fat Zonal level, 2. Shortage of CTC Materials, 3. Shortage of Water - Guard
30	Major areas support from the government	1. Budget Support for emergency preparedness, 2. Drugs for emergency like for AWD, 3. CTC Materials Support for AWD and, 4. Water Purification (water-guard), for AWD.

Contingency Stock Estimation at East Harerege Zone selected Districts as of July, 2010

	Contingency Stock Estimation				
	Districts				
Items	Girawa	Kurfa-Chelle	Gola-Oda	Bedeno	Gursum
ORS Packet	20 boxes	10 boxes	2 boxes	76 bag	600 Sackets
Ringer's Lactate 1Liter	10 bags	13 bags	48 bags	22 bags	151 bag
NG tube	none	10 Pcs	15 pcs	none	846 pcs
Doxycycline, 100mg	3 Tins	2 TINS	2 Tins	none	18 pack
Erythromycin 250 mg (Children)	0	0	0	0	Na
Ciprofloxacin	0	0	0	0	10 pack
Amoxicillin 500mg	500 capsules	420 capsules	4 boxes	1061 capsules	5 boxes
Cotrimoxazole (480mg)	5 TINS	3 TINS	2 Tins	10 Tins	23 boxes
Coartem	7 boxes	10 boxes	5 boxes	2 boxes	12 boxes
Crystalline Penicillin	0	0	0	none	Na
AC-Vaccine	0	0	5 boxes	none	Na

Background Information - West Harerege Zone

West Harerege Zone is one of the Oromia regional states found in the eastern part of the region bordering 4 Oromia Zones (East Shoa, East Harerege, Arsi and Bale) and two regions: Somali and Afar regional states. Asab-Teferi (Chiro) is the Capital of the zone and the total area of the zone is 217,230 Km².

The Zone has a total of 14 districts with two towns (Chiro and Bedessa).

Demographically, the zone has a total population of 2,045,690 with its annual population growth rate 2.9%. The Male to Female sex ratio is 49:9:50. and 92% of the population is live in the rural area. The population density of the zone is 108 persons per square km.

From the total land, 15.2% covered by Highland, 41.38% mixed and the remaining 43.6% covered by low-land areas.

Agriculture is the backbone of its economy and means of livelihood for more than 90% of the population. The main crops in the zone are Coffee and Khat (A Stimulant Shrub). Coffee is the major cash crops in the zone.

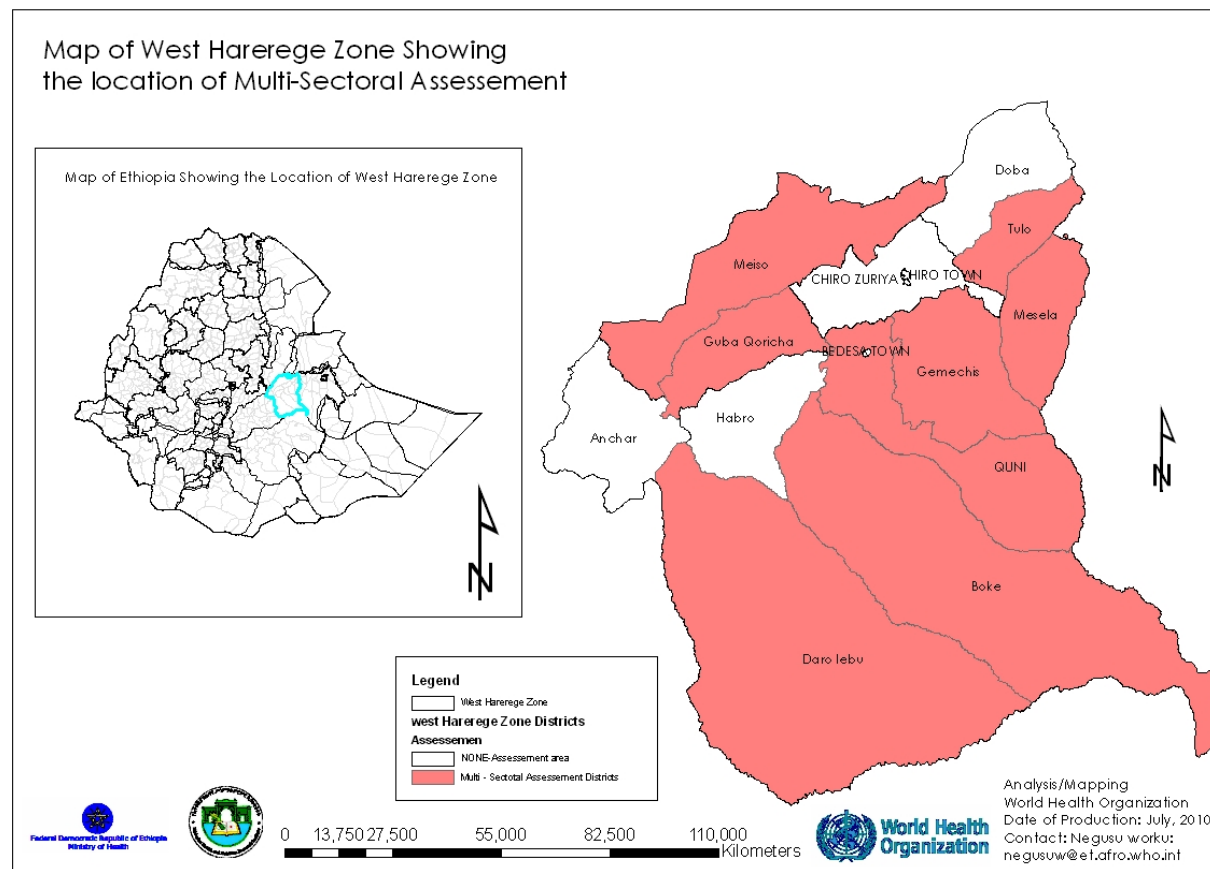


Fig xx showing the Selected visited districts during the multi-sectoral assessment

Ten Top diseases in West Harerege Zone from July 2009 – March, 2010

No	Ten Top Diseases in West Harerege Zone
1	Soft Injury Purposely Infected By Other Persons
2	Acute Update Respiratory Infections
3	All Malaria Diseases
4	All Helmenthasis
5	Gastritis and Doudenitis
6	Primary typical and other un specified Pneumonia
7	All Diarrhea Diseases
8	All Diseases of jento Urinary System
9	Anemia
10	Infections of Kidney

Distribution of Human Resources, Public and Private Health Facilities in West Harerege Zone

Human Resource of Health - West Harerege Zone	
Profession	No
MD + Specialist	5
General Practitioner (MD)	13
Health Officer	66
BSc. Nurses	13
BSc. Env.Health	28
BSc. Aesthetics	3
Clinical Nurse	256
BSc. Laboratory Technicians	16
Pharmacy Technicians	32
Midwife Nurse	33
Sanitarian dip	36
Health Assistance	26
Health Extension Worker	820
Total	1347

Distribution of Public Health Facilities	
Hospital	2
Health Centers Type A	9
Health Centers Type B	31
Health Post	295
Total	337

Distribution of Private Health Facilities	
Type of Health facilities	No
Higher Clinic	1
Medium Clinic	10
Lower Clinic	46
Pharmacy	1
drug Shop	16
Drug Vender	42
Total	

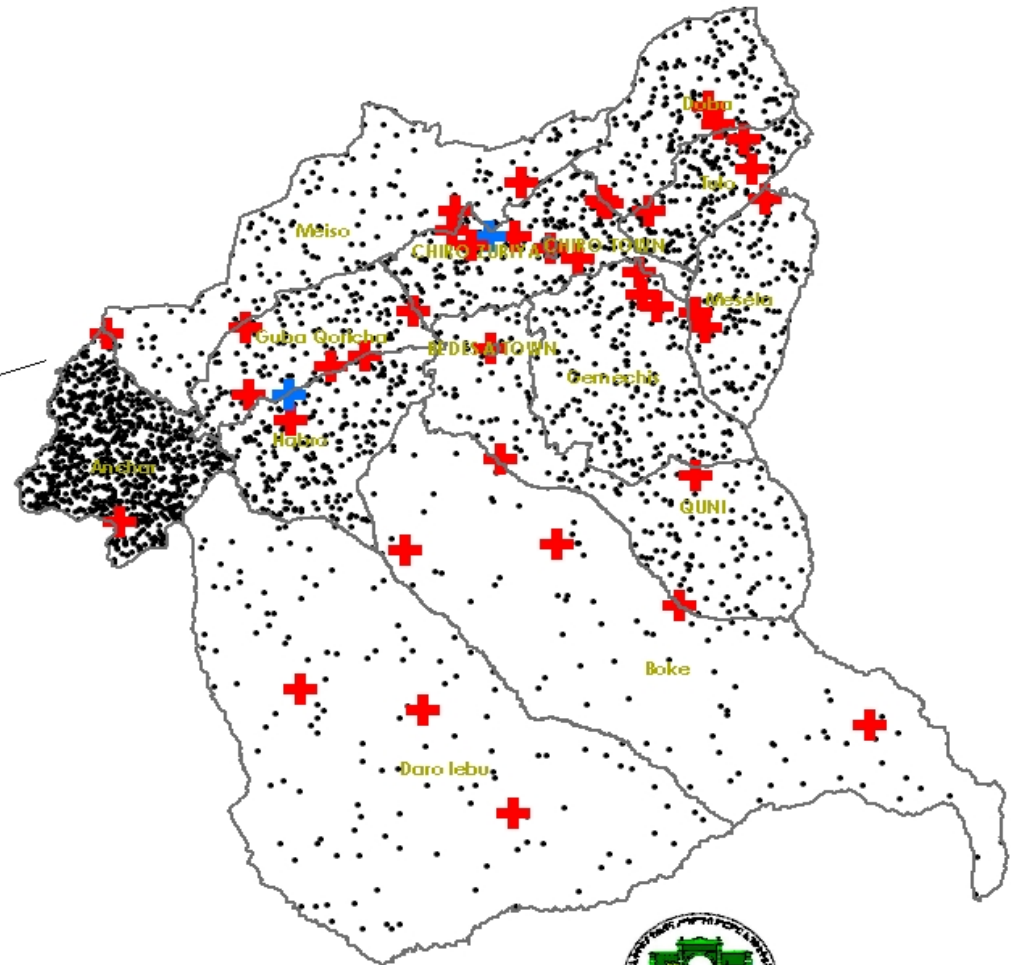
Source: West Harerege Zonal Health Office

Map of West Harerege Zone Showing Population Density with Health Facility Distribution

Legend

- 1 Dot = 1
- Hosp
- HCs
- 1 Dot = 1,000
- Popn_1
- NWHZB Zonal Boundary

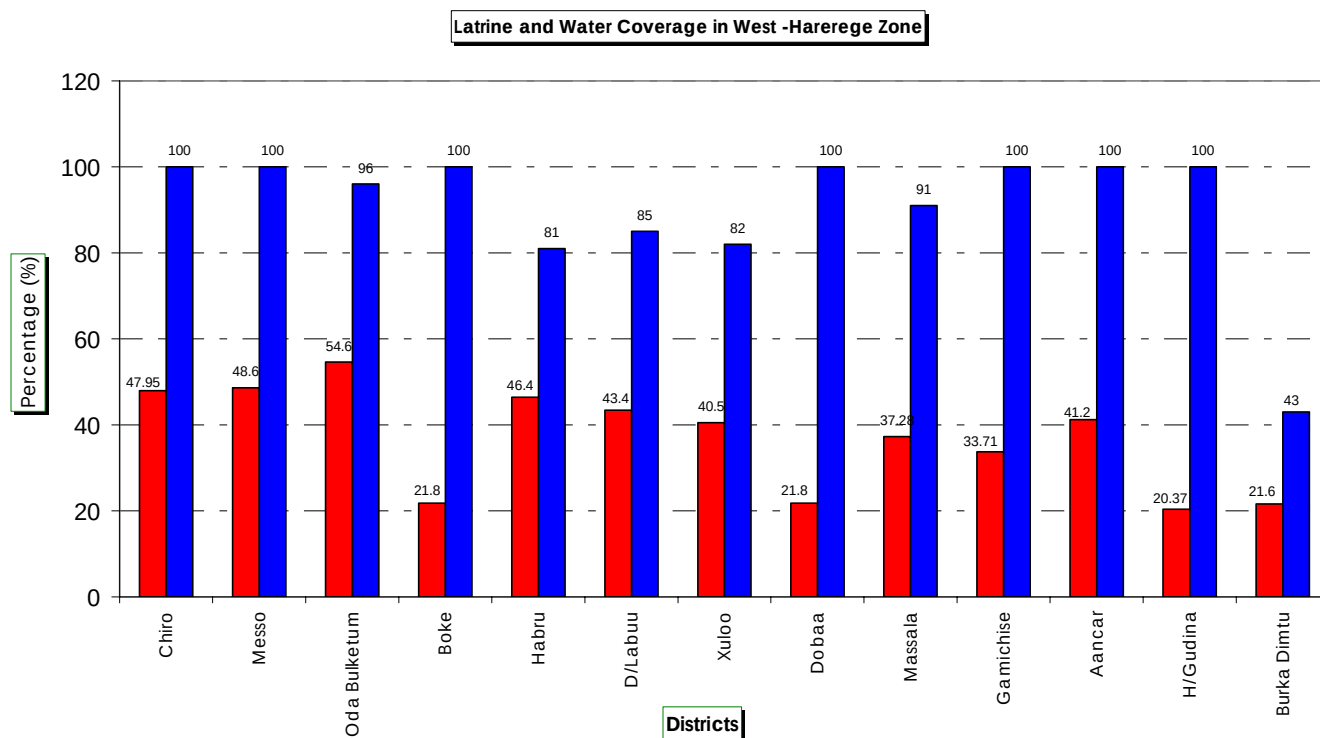
Map of Oromia Region Showing the Location of West Harerege Zone



Analysis/Mapping:
World Health Organization
Ethiopia Country Office
Emergency and Humanitarian Action Unit
Data Source: West Harerege Health Bureau
Contact: Negusu Worku: negusuw@et.afro.who.int
Date of Production: July, 2010



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Analysis: World Health Organization
 Ethiopia Country Office
 Data Source: West Harerege Zone Health Office
 Date of Production: July, 2010
 Date of Production: July, 2010



Water Coverage Latrine Coverage

According the official data from the West Harerege Zonal health office depicting that, the zone water and latrine coverage is 47.5% and 93%. There are six districts that has been marked as a *Chronic shortage of water supply including: Daro-Lebu, Hawi-Gudina, Boke, Burka-Dimtu and Messo. (Please see the map above).* The remaining districts have low water coverage.

Table xxx Shows AWD affected districts in West Harerege Zone from July, 2009 – October - 2010

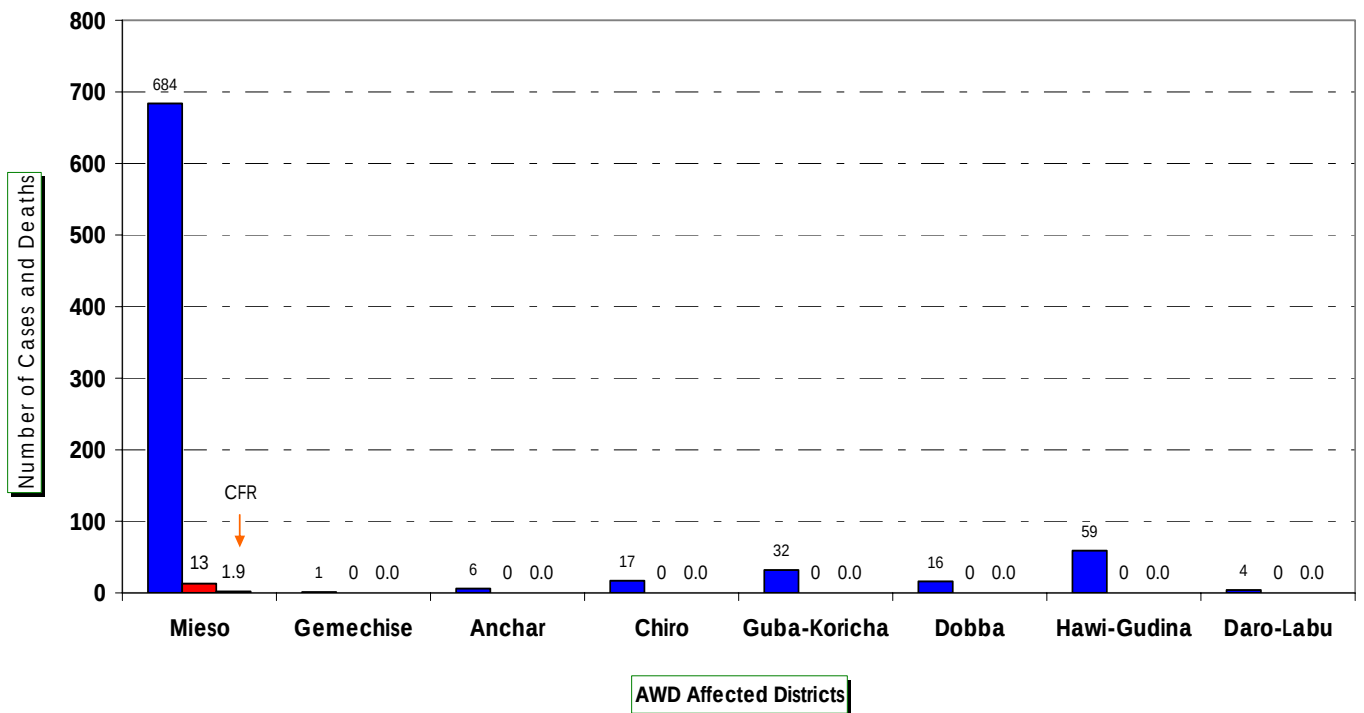
Districts	Cases	Deaths	CFR
Mieso	684	13	1.9
Gemechise	1	0	0.0
Anchar	6	0	0.0
Chiro	17	0	0.0
Guba-Koricha	32	0	0.0
Dobba	16	0	0.0
Hawi-Gudina	59	0	0.0
Daro-Labu	4	0	0.0
Total	819	13	1.6

Health Update in West Harerege Zone

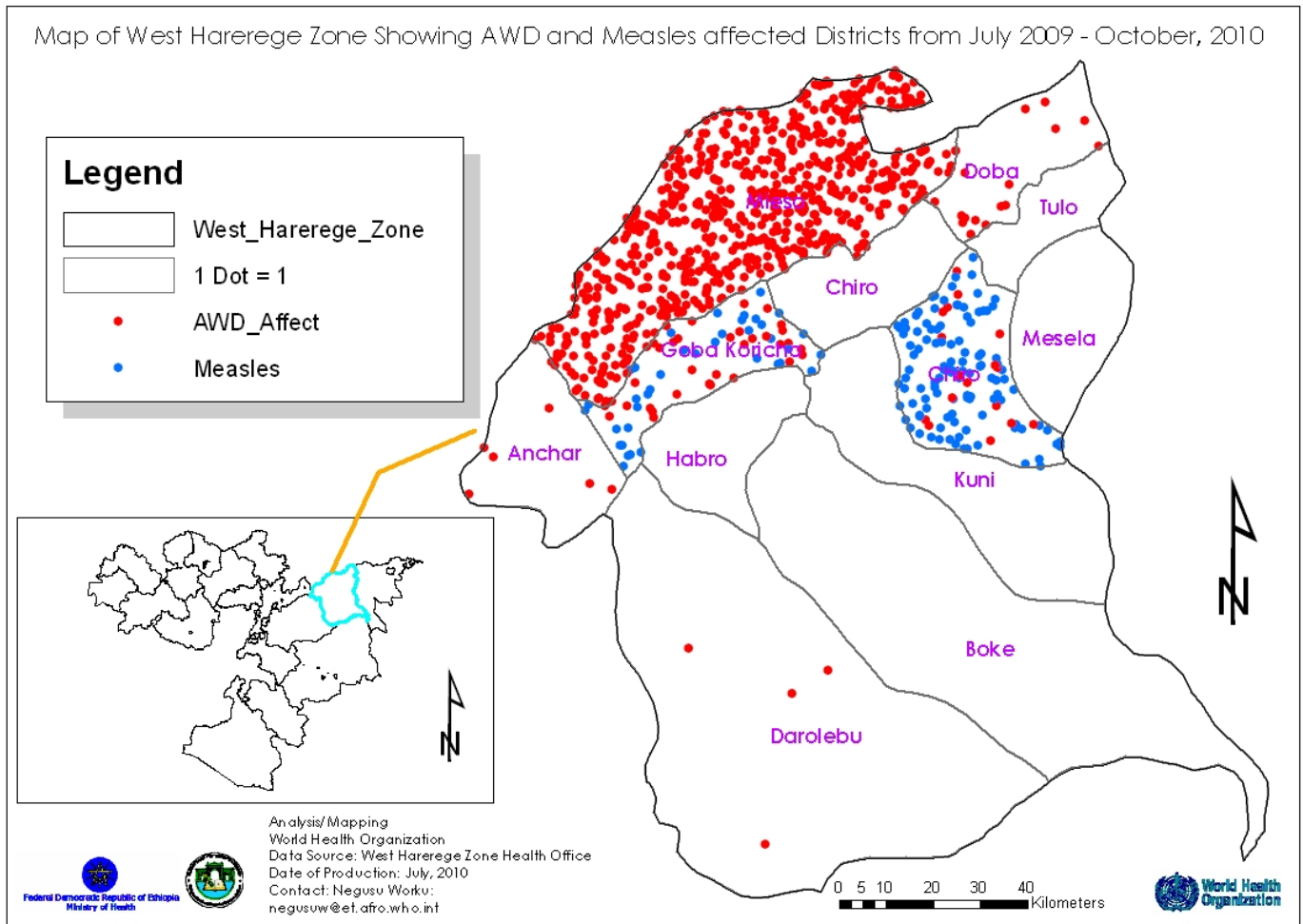
Primary Health Care [PHC] Coverage in the West Harerege Zone achieved 73.3%. It has a total of 2 Hospitals, 40 Major Health Centers, and 295 health post. The Zone has a total of 1347 health professionals. The ratio of one Medical Doctor and Nurses to the zone is 113,649 and 4019 respectively.

In the West Harerege zone eight districts were affected by AWD from July 20, 2009 – October, 2010 with a total case of 819 with 13 Deaths and CFR of 1.6.

AWD Affected Districts in West Harerege Zone from
July, 2009 - October, 2010, Oromia Region, Ethiopia



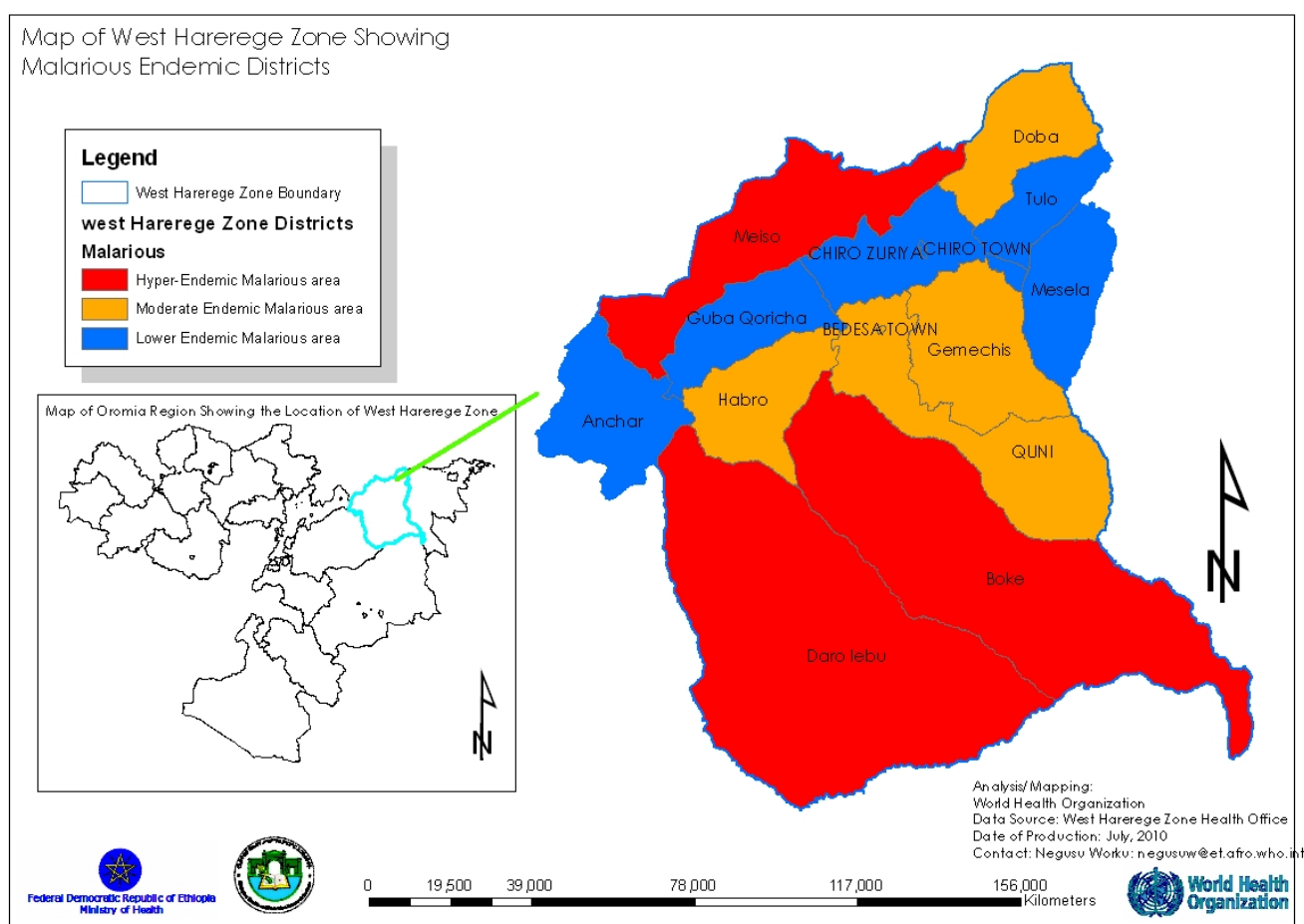
Analysis: World Health Organization
Data Source: West Harerege Zone Health Office
Date of Production: July, 2010
Negusu Worku: negusuw@et.afro.who.int



The first index case was reported *Mull Kebele river in Somali region*, a place where both Communities from Oromia and Somali accessing the same water sources. By then, as reported from the zonal health bureau technical group, the water source was contaminated and it had been expanding to eight kebeles of West Harerege zone.

Malaria – in West Harerege Zone

In West Harerege Zone Six Woredas have been associated with a hyper-endemic districts including: **Me-esso**, **Daro-lebu**, **Hawi-Gudina**, **Boke**, and **Burka-Dimetu**, five districts are labeled as *moderate malarious endemic* area including: **Doba**, **Habro**, **Oda-Bultim**, **Bedessa Town** and **Gemechise** and low malarious area are **Tulo**, **Guba-Koricha**, **Chiro-Rural**, **Chiro-Town**, **Messela** and **Anchar**.
[Pls. see the map below].



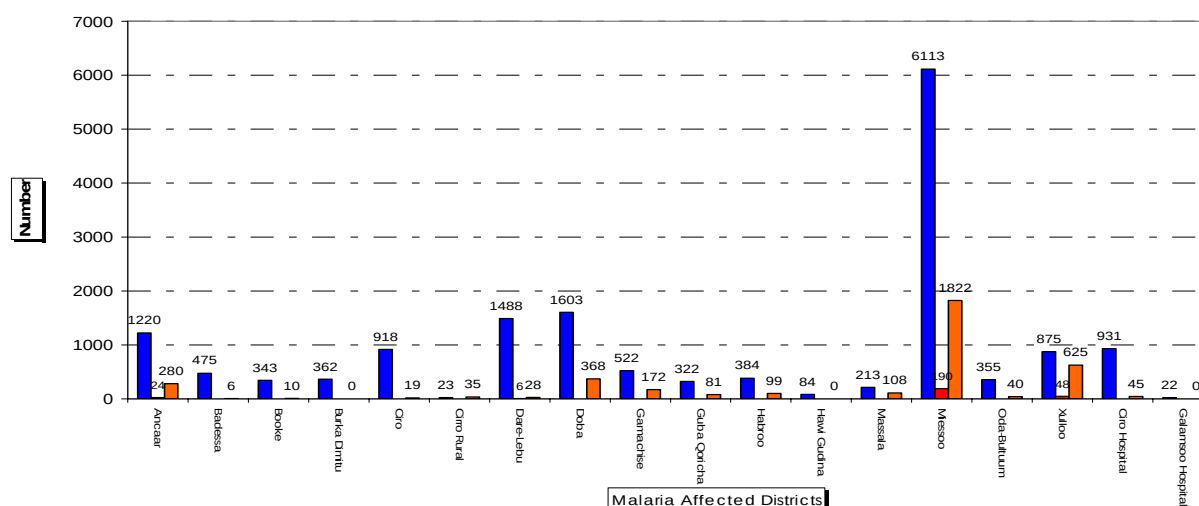
Source of Data: West Harerege Zone Health Office

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Table xxxx Shows Malaria Confirmed Clinically, RDT and by Laboratory from West Harerege Zone

Districts	Clinically Confirmed Malaria Cases	RDT	Lab Confirmed
Ancaar	1220	24	280
Badessa	475	0	6
Booke	343	0	10
Burka Dimitu	362	0	0
Ciro	918	0	19
Cirro Rural	23	0	35
Dare-Lebu	1488	6	28
Doba	1603	0	368
Gamachise	522	0	172
Guba Qoricha	322	0	81
Habroo	384	0	99
Hawi Gudina	84	0	0
Massala	213	0	108
Miessoo	6113	190	1822
Oda-Bultuum	355	0	40
Xulloo	875	48	625
Ciro Hospital	931	0	45
Galamsoo Hospital	22	0	0
Total	16253	48	3738

Distribution of Malaria Cases from July, 2009 - June, 2010 in West Harerege Zone, Oromia Region, Ethiopia: July, 2010

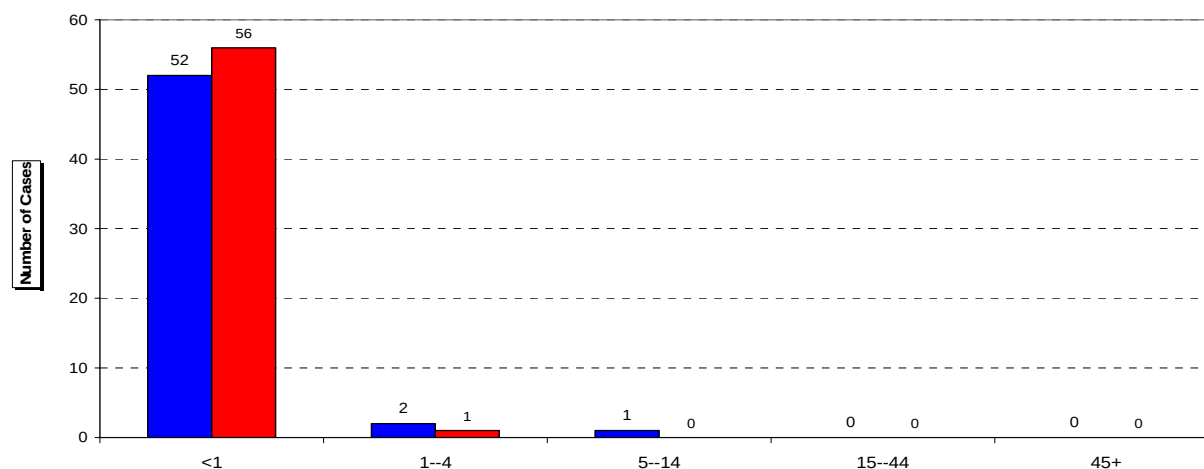


Analysis: World Health Organization
 Data Source: West Harerege Zone Health Office
 Date of Production: July, 2010
 Negusu Worku: negusuw@et.afro.who.int

Status of ITNs Distribution in West Harerege Zone			
Districts	Plan	Achieve	%
Ancaar	16066	16000	99.59
Badessa	9251	0	0
Booke	22975	0	0
Burka Dimitu	13975	0	0
Ciro Rural	32460	32000	98.58
Cirro Town	15970	0	0
Dare-Lebu	46064	0	0
Doba	16810	0	0
Gamachise	24733	0	0
Guba Qoricha	13975	0	0
Habroo	29858	0	0
Hawi Gudina	13132	0	0
Massala	18132	18100	99.82
Miessoo	26359	26400	100.16
Oda-Bultuum	19896	0	0
Tulloo	19984	1300	6.51
Total	339640	93800	27.62

The Zone is known by one of the Malarious area. 1.67 million Population is at risk of malaria in the West Harerege Zone. Despite the first round bed net coverage and distribution was successful, the replacement process has been delayed and the data shows it is only 27.62%. There is fear of Malaria outbreak by the Belg Season rain and may aggravate the situation. Out break is expected in the low-land areas of both East and West Harerege Zone as reported from the Zonal health desk and Zonal officials. ITNs replacement distribution has to be made before September and October, 2010. This is mainly because of the fact that, after the rainy season, another pos t- summer (kiremet) Malaria outbreak will occur.

Malaria Prevalence in West Harerege from July 2009-March			
District	Malarious Population	Cases	Cases/1000
Ancaar	87363	1229	14.07
Badessa	23127	343	14.83
Bookee	94032	299	3.18
Burka Dintuu	75732	236	3.12
Cirro	124236	702	5.65
Ciro Town	39924	856	21.44
Dare-Labuu	119200	1344	11.28
Doba	115289	1389	12.05
Gamachise	139479	568	4.07
Guba Koricha	109895	371	3.38
Habro	152517	357	2.34
Hawi Gudina	82870	84	1.01
Messela	93741	270	2.88
Miessoo	153380	7143	46.57
Oda-Bultuum	135710	311	2.29
Tullo	122791	1254	10.21
National	1669286	16756	10.04

Distribution of Measles Cases in West Harerege Zones as of May, 2010

Age Group vs Sex

Analysis: World Health Organization
 Data Source: West Harerege Zonal Health Office
 Date of Production: July, 2010
 Contact Person: negusuw@et.afro.who.int



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Nutrition in west Harerege Zone from September 2009 – January, 2010

SAM Cases Management Performance Report of OTP from September 2009-January 2010

District	Beginning of the Month	New Admission					Discharge						Transfer		Total Discharge	Total Cases at the end of the Month
		MUAC<11	OEDEMA	RELAPSE	RE-ADMISSION	Transfer from Another	Cured	Death	Unknown	Defaulter	Non-Responding	Medical Transfer	Transfer out to OTP	Transfer Out		
Anchar	397	642	772	4	0	0	419	1	0	40	0	0	23	4	487	1328
Boke	404	161	312	0	0	0	799	0	0	0	0	0	0	3	802	75
Bu/Dhintu	203	73	2	0	0	0	286	0	0	4	8	0	0	0	298	-20
Chiro	476	402	108	0	0	0	893	0	0	0	0	0	12	0	905	81
Dara-Labu	199	197	219	6	0	0	460	0	6	5	9	0	3	4	487	140
Doba	531	174	96	0	0	6	612	0	0	0	0	0	0	0	612	189
Gemechise	545	257	183	0	0	0	616	0	0	2	0	0	2	0	620	365
Guba Koricha	111	226	85	2	20	0	258	0	5	0	0	0	1	4	268	176
Habro	835	600	169	0	0	0	1294	0	0	1	2	2	0	1	1300	304
Hawi Gudina	226	91	53	0	2	0	247	0	18	0	0	0	0	0	265	105
Oda/Bultum	202	242	210	0	0	0	532	0	13	1	1	0	3	4	554	106
Messela	668	440	259	9	0	0	155	1	323	48	3	0	57	8	595	807
Mieso	1021	1727	59	0	0	6	2050	0	9	9	0	0	1	0	2069	738
Tulo	118	174	106	0	0	0	195	0	62	24	0	0	2	2	285	119
Chiro City	0	0	0	0	0	4	0	0	0	0	0	0	0	0	0	0
Bedeno City	26	0	0	0	0	0	8	0	12	0	0	0	0	0	20	6
Chiro Hospital	0	11	9	0	0	0	15	0	0	1	1	5	0	0	37	-17
Gelemeso Hospital	7	3	5	0	0	0	0	0	0	0	0	0	0	0	0	15

Source: West Harerege Zonal Health Bureau

Monthly Statistics Report - Management of Sever Acute Malnutrition-Therapeutic Feeding Programme																
	New Admission						Discharge									
Age Group	MUAC<1	OEDEMA	Relapse	Re-admission	Transfer in from another	Total admissions	CURED	Death	Unknown	Defaulter	Non-Responder	Medical Transfer	Transfer OUT	Transfer OUT	Total discharge	Total end of the month
<6 Mo	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0
6–59m	6645	1114	209	30	38	8036	5154	2	29	42	23	8	126	42	5426	2610
5 – 10y	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0
10-17y	0	0	0	0	0		0	0	0	0	0	0	0	0	0	0

Data Sources: West Harerege Zone Health Office

Table xxx Shows the Distribution of HEW and the Number of OTP Services							
District	District Population	No of Kebeles	No of HEW	No HPs	No HC	No HC Running OTP	No Hos
Anchar	91867	27	42	16	1	1	0
Bedeno Town	20867	2	0	0	1	1	0
Boke	88967	23	42	22	3	1	0
Bu/Dhintu	80218	22	15	9	2	2	0
Chiro Zuria	184479	39	78	32	3	3	0
Chiro Town	38503	3	6	3	1	1	1
Oda/Bultum	172302	37	72	35	3	3	0
D/Labu	126989	40	51	36	3	3	0
Doba	145264	41	62	40	4	4	0
Gemechise	199581	32	57	31	3	3	0
Guba Koricha	130100	30	50	26	4	4	0
Habro	203069	37	59	32	2	2	0
Hawi Gudina	90300	32	47	16	0	0	1
Mesella	126046	26	44	25	3	3	0
Mieso	142867	49	87	45	4	4	0
Tulo	159511	33	57	30	3	3	0
Total	2000930	502	769	398	40	38	0

Data Source: West Harerege Zonal Health Office

Result – West Harerege Zone

Mesela – District General Background and Current Situational Analysis

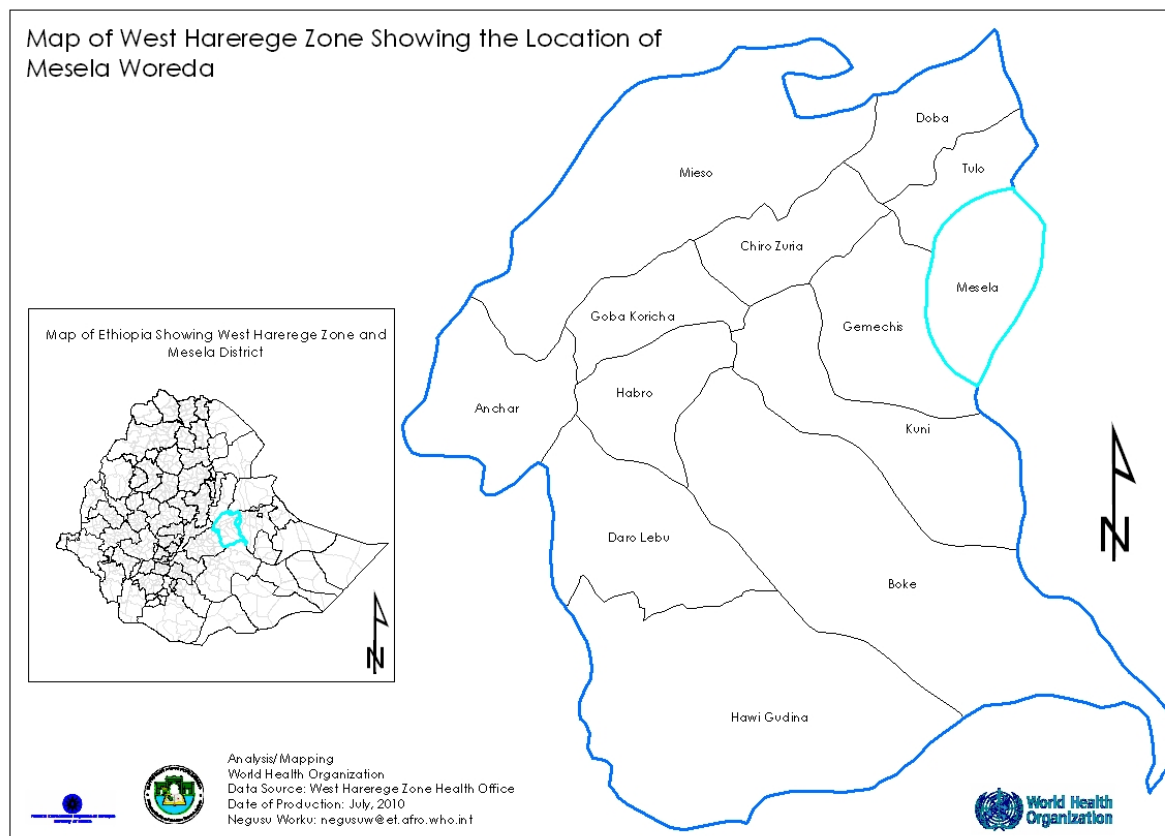
Mesela District is Located in the North East of West Harerege Zone. It has a total of 25 rural peasant association (PAs) with a population of 162,000. The District has one major health center and 2 under-constructions Type B health center with a total of 50 health extension workers.

The Primary Health care and water coverage at the district is 86% and 37.2% respectively.

Because of the *Belg Season* heavy rain and crops “WAG” disease, massive potato, Sorghum, Wheat, and Maize Production had been totally damaged.

Health Status

During our visit in the district, we were told that, there is no as such communicable diseases outbreak. Only Spot Malaria cases were reporting from hyper-endemic Kebeles. Among the total 25 Kebeles, 15 of them are labels as *hyper-endemic districts*, 5 of them are moderate and the remaining are less.



Nutrition

An official report from the district data indicating that, 450 children are on 18 OTP sites and in 2 Stabilized Center (SC).

527 Children are in need of additional supplementary food.

Ten Peasant Associations (PAs) doesn't have water sources in the ground and the community at large suffering by water shortage. These PAs are: ***Gebise, Gufensizte, Dekukese, Kebesso, Aba-Chibessa, Ere-Kobase, Abdir, Hadekema, Shammee and Selama.***

Tello Current Situational Analysis

- * Tulo district is located in the North Eastern part of the West Harerege Zone,
- * Total population as of CSA 2007 - 148,702
- * Water and Latrine Coverage in the district is 27% and 97% respectively,
- * Water Contamination problem seen at **Edo Base** kebele,
- * It is a UNICEF Learning Woreda,
- * There is no outbreak of communicable diseases
- * The district has a total of 30 PAs and 10 of them are under a serious water problem,
- * Among the total PAs 43% of the district is at risk of Malaria,
- * *47% of the total crops had been damaged because of the heavy rain flooding,*
- * *Livestock and Hen's disease killed a number of cattles and hen's,*
- * *as a result of the heavy rain:*
 - * *17985 population are in need of a desperate food,*
 - * *15,740 in emergency food need,*
 - * *The district has a total of 58 health Extension Workers and 30 Health Post,*

Goba Koricha Current Situational Analysis

- Excess rain was registered. As a result it damaged belg crops at flowering stage,
- Problems of water and grazing field for cattles are noted,

Health

- There was a Measles out break on May and 38 cases were reported,
- AWD – controlled but there is fear from the Me-sso districts,
- All health post established OTP,
- There is 1 major health center,
- 3 Type A health centers,
- 10 health post
- Water and Latrine coverage is 25% and 90% respectively,
- IMC international organization supporting the districts on Nutrition program,
- Among the total 31 PAs – 19 of them are labels as chronic shortage of water supply,

Messo – District Current Situation Analysis

- Location North Western of the zone,
- Water Coverage is 23%
- There is high is AWD because of the river that has been repeatedly contaminated at the border of Somali region,
- It has a total of 49 PAs of which 23 PAs are hyper-endemic malarious kebeles,
- ITNs has not been replaced,
- The water Schemes in the PAs damaged by flood,

In both **West and E/Harergie Zones**: there is a multi-hazard approach and multi-sectoral horizontal coordination forum between sectors and NGOs; chaired by the head of zones that have conducted a regular meeting. But, zonal health departments have no preparedness fund at in order to respond on timely basis for major gaps in their vicinity; eve if they had less amount of drugs in stock; as shown below:

zone	supplies	Amount/unit	Population	remark
E/Harergie	coartem	207 boxes	2,980,862	
	Doxcycycline	103 boxes		
	ORS	330 boxes?		
	Ringer lactate	71 cartone		
W/Harergier	coartem	106 box	2, 045,689	
	Doxcycycline	514 pk		
	ORS	4800 sachets		
	CTC	1		

The major challenge for both zones , in order to effectively run their mission were due to :

- lack of budget for capacity building with regard to public health emergency management tasks
- shortage of supplies like CTC materials, water guards
- scarcity of logistic and transportation

In order to full fill the gaps that the zones have, they need compulsory support from RHB, MoH and any partners who are working for health.

Belg – Multi – Sectoral Humanitarian needs assessment from June 22 – July 12, 2010: By Negusu Worku GEBRMICHAEL [negusuw@et.afro.who.int]

Summery of West Harerege Zone	Visited Districts in the West Harerege Zone during the Assessment		
Major Indicator Variables	Oda Bultum	Daro Lebu	Boke
Measles Vaccination in u-1%	78%	na	81
Pentavalent Vaccination in u-1%	98%	na	88
ITNs Coverage%	0 (less malarious district)	na	na*
Woreda IDSR timeliness rate L6months%	90	95	90
% of District Popn using improved Water	65	na	60
% of District Popn getting water from treated sources	1.1	na	35
Latrine Coverage%	79	85.6	36.6
Latrine Utilization%	74	42	20
Health Partners in the Woreda	none	GOAL	GOAL
Population	151,066	126,424	95,903
U5	24,872	20734	15728
Above 5	126789	105,690	80,175
Water Coverage	54.5	43.4	21.8
Free Health care Services	ALL Medical Services	ALL Medical Services	ALL Medical Services
Waiver System Covers	ALL Curative Services	ALL Curative Services	ALL Curative Services
Top 5 Diseases	RTI, Diarrhea, IP, RI, AFI, Skin Diseases	Thypoid, Dysentery, Diarrhea, Malaria	AURI, Helments, Diarrhea, Injuries, Anemia
Nutrition			
Total SAM Cases as of 2010	537	na (vail at the GOAL office)	592
Total no of TFP(OTP/SC)	185	na (vail at the GOAL office)	115
Number of New Cases SC	12	na (vail at the GOAL office)	35**
Number of New cases OTP	737	na (vail at the GOAL office)	
Total no of OTP/SC for this Month (Jan--May, 2010)	167	na (vail at the GOAL office)	51

* ITNs has been secured but not yet distributed

** Data not available for the month April and May

Summery of East Harerege Zone	Visited Districts in the West Harerege Zone during the Assessment		
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Belg – Multi – Sectoral Humanitarian needs assessment from June 22 – July 12, 2010: By Negusu Worku GEBRMICHAEL [negusuw@et.afro.who.int]

Major Indicator Variables	Mesela	Goba-Koricha	Me-sso	Tillo
Measles Vaccination in u-1%	83%	na	60	86
Pentavalent Vaccination in u-1%	101%	na	87	97
ITNs Coverage%	na*	96	0	na
Woreda IDSR timeliness rate L6months%	75	100	NA	100
% of District Popn using improved Water	na*	25	0	55
% of District Popn getting water from treated sources	na*	na	0	30
Latrine Coverage%	75	na	98	84
Latrine Utilization%	62	na	60	54
Health Partners in the Woreda	IMC	IMC	IMC	WV, IMC
Population	162,046	122,521	153,380	159,510
U5	26,128	20092	5368	26319
Above 5	135918	102,429	148,012	133,191
Water Coverage	37.3	24.1	48.6	40.5
Free Health care Services	ALL Medical Services	ALL Medical Services	ALL Medical Services	ALL Medical Services
Waiver System Covers	ALL Curative Services	ALL Curative Services	ALL Curative Services	ALL Curative Services
Top 5 Disease	Pnemnia, Malnutriton, IP, Diarrhea, Gastrists	Injury, PUD, Malaria, Anemia, AGN	Malaria, Malnutrition, Pnemonia, Anemia, Skin Infections	URTI, GASTRICS, MALARIA, ANEMIA, INJURY
Nutrition				
Total SAM Cases as of 2010	46	251	3964	848
Total no of TFP(OTP/SC)	10	30	184	848
Number of New Cases SC	0	3	0	10
Number of New cases OTP	277	0	1082	848
Total no of OTP/SC for this Month (Jan--May, 2010)	90	33	184	848

Water, Sanitation and Hygiene (WASH)

1. Introduction

Without adequate water and sanitation and access to basic health care, populations are often more directly and immediately at risk of dying from water related disease outbreaks. Moreover, all stakeholders have agreed that a greater focus on non food interventions is needed to make food aid more effective, and also to identify contexts where other types of interventions may be more appropriate. Until recently, the role water plays in household income generation (or productive) activities that require water, such as livestock raising, small plot irrigation etc are less acknowledged. However, it is well understood that securing adequate water for human consumption is essential for human survival.

Accordingly, the assessment of Water, Sanitation and Hygiene (WASH), Health, Nutrition and Education situation with respect to humanitarian needs and planning was undertaken in Oromia region 2 zones (east Hararge and West Hararge) by the team composed of UNICEF, WFP, WHO, USAID, CARE, Federal DRMFSS, Oromia DPPC and Oromia Health Bureau in which both food and non food components are covered..

Note: In this report, only the WASH related issues, on which I am working, are taken for the monthly activity report. The overall findings/reports of the belg assessment will be shared after debriefing at regional and federal level.

1. Summaries of Key findings and recommendations

- In both east and west Hararge zones, the onset of the ‘belg’ rain was earlier than the last consecutive 4 years and the rainfall pattern (distribution and amount) was good.
- In chronically water shortage areas, the existing ponds have harvested adequate water which may last for more than two months after the stop of rainfall.
- In contrary to the other year, in some woredas (Mieso, Boke, Daro Labu) heavy rainfall was reported in April, which have resulted to damage to large ponds and existing water schemes. On the other hand, in Messela and Tulo woreda of WestHararge zone, increases in the yield of springs are reported. Generally the quantities of water availabilities for human and livestock consumptions are increased.
- In both zones, there is no WASH related emergency for the time being, however, in chronically water shortage areas like Midhaga Tola, Mayu Muluke, Kumbi and Babile woredas of east Hararge zone and Daro Labu, Hawi Gudina, Boke Burka Dhimtu and Mieso woredas of west Hararge zone needs close monitoring of the water availability after the stop of the main rainfall. Even though, the ponds have harvested adequate water for the time being;
- due to heavy rainfall, the ponds capacity may have reduced due to siltation,
- runoff flood has damaged some of the ponds like in Mieso woreda that can increase concentration of people and livestock on the remaining water points / ponds,
- The water can be lost in the form of evaporation and infiltration.
- If there is no adequate rain fall of the main rain and the existing water in the ponds deplete, **196,832 people** (91,830 east Hararge

zone 7 woredas and 105,002 west Hararge zone 5 woredas of zones) that have chronic water problem will be at risk of acute water shortage which needs close monitoring. About **ETB 4,828,961** is required for water tankering for 3 months.

- There are about 1,104 water schemes (842 functional and 262 non functional) in East Hararge and 512 water schemes (409 functional and 103 non functional) in west Hararge zones. The non functional water schemes accounts 24% and 20% of the total schemes in east Hararge and west Hararge zone respectively. The non functional water schemes have aggravated the shortage of potable water supply in the zones:
- Most of the non functional water schemes that can be maintained at woreda level are due to shortage of budget. Most of the water schemes don't have strong WASHCOs. Water schemes that don't have WASHCOs are suffering from lack of budget for maintenance. But Water schemes which have strong WASHCO, collect water fees and deposited in the bank account which they can use for maintenance and expansion works. Hence, establishing and strengthening of WASHCOs for each water schemes will ensure the ownership/ functioning of the water schemes on sustainable basis.
- For short term response; financial support is required for maintenance of the non function water schemes.
- According to the report from zone water offices, 51.3% and 54.3% people in east and west Hararge zones respectively are using unprotected water sources. These people are using mainly runoff water; hence, supplies of water treatment chemicals are required to treat the water at household level.
- Speedup of the completion of the ongoing water supply projects in the zones will contribute to alleviate the water shortage.

- On the other hand, there is no school or health institutions closed due to WASH related emergency.

2. East Hararge zone

4.1 Key Findings

- In east Hararge zone, the on set of the ‘belg’ rain was earlier than the last consecutive 4 years and the rainfall pattern (distribution and amount) was good.
- The existing ponds in Midhaga Tola, Mayu Muluke, Gola Oda, Kumbi and Babile woredas are harvested adequate water which can serve for more than two months after the stop rainfall.
- There is no WASH related emergency for the time being, however, in chronically water shortage areas like Midhaga Tola, Mayu Muluke, Kumbi and Babile woredas of east Hararge zone needs close monitoring of the water availability after the stop of the main rainfall. Even though, the ponds have harvested adequate water for the time being;
 - ✓ Due to heavy rainfall, the ponds capacity may have reduced due to siltation,
 - ✓ Runoff flood has damaged some of the water schemes like in Melka Belo woreda, one on spot spring is damaged that can increase concentration of people and livestock on the remaining water points / ponds,
 - ✓ The water can be lost in the form of evaporation and infiltration.
- If there is no adequate rain fall of the main rain and the existing water in the ponds depilate, 96,830 people (in Midhaga Tola, Mayu Muluke, Kumbi, Gursum, Chinaksen and Babile woredas) that have chronic water problem will be at risk of acute water shortage.

- There are about 1,104 water schemes (842 functional and 262 non functional) in East Hararge zone. The non functional water schemes accounts 24% of the total water schemes. The non functional water schemes have aggravated the shortage of potable water supply in the zones:
 - Most of the non functional water schemes that can be maintained at woreda level are due to shortage of budget. Most of the water schemes don't have strong WASHCOs. Water schemes that don't have WASHCOs are suffering from lack of budget for maintenance. But Water schemes which have strong WASHCO, collect water fees and deposited in the bank account which they can use for maintenance and expansion works. Hence, establishing and strengthening of WASHCOs for each water schemes will ensure the ownership/ functioning of the water schemes on sustainable basis.
 - For short term response; financial support is required for maintenance of the non function water schemes.
 - According to the report from zone water offices, 51.3% people in east Hararge zone are using unprotected water sources, mainly runoff water hence, to treat the water at household level, supplies of water treatment chemicals are required.
- Speedup of the completion of the ongoing water supply projects zones will contribute to alleviate the water shortage.
- On the other hand, there is no school or health institutions closed due to WASH related emergency.

4.2 WASH

This year the ‘belg’ rain was started earlier than the other year, distribution was uniform and amount varies from normal to above normal. In April and May there was heavy rainfall (above normal) that has resulted to damage of crop lands in Deder, Bedeno, Melka Balo, Goro Gutu and Kombolcha woredas. In low land woredas (Midhaga Tola, Mayu Muluke, Gola Oda, Kumbi and Babile woredas) that have chronic water problems, ponds have harvest adequate water that could last for more than two months after the end of the rainy season. The belg rain has improved the quantity of water availability. On the other hand, According to the report from zone water offices, 51.3% of people in east Hararge zone are using unprotected water sources, mainly runoff water. In the visited woredas like: Kurfa Chale, Girawa, Gola Oda, Bedeno, Midhaga Tola and Meta, water born and water scarce disease like skin disease, eye disease and diarrhea diseases are among the ten top disease. Hence, to treat the water at household level, supplies of water treatment chemicals are required.

Based on information obtained from zone water office, there are about 1,104 water schemes of which 842 functional and 262 are non functional due to low yield and mechanical problems. The non functional water schemes accounts to 24% of the total water schemes, which have aggravated the shortage of potable water supply in the zone. Most of the mechanical problems that can be maintained at woreda level are due to shortage of budget. The majority of the water schemes don’t have strong WASHCOs. Water schemes that don’t have WASHCOs are suffering from lack of budget for maintenance. But Water schemes which have strong WASHCO, collect water fees and deposited in the bank account which

they can use for maintenance and expansion works. Hence, establishing and strengthening of WASHCOs for each water schemes will ensure the ownership/ functioning of the water schemes on sustainable basis.

Table 1: Summaries of the status of existing water Schemes in east Hararge zone

No.	Type of scheme	Functional	Non functional	Total
1	Motorized BH	74	33	107
2	SW with hand pump	313	104	417
3	Hand dug well	219	28	247
4	protected on spot spring	114	52	166
5	Spring with distribution system	122	45	167
	Total	842	262	1,104

There is no WASH related emergency for the time being, however, there are pocket areas that have chronic water shortage like in Midhaga Tola, Mayu Muluke, Kumbi, Gursum, Chinaksen and Babile woredas of east Hararge zone which needs close monitoring of the water availability after the stop of the main rainfall. Even though, there was adequate rainfall and there are water in the ponds for the time being, the pond water may not last longer due to:

- The ponds capacity may have reduced by siltation as a result of heavy rainfall. This needs further investigation.
- Runoff flood has damaged some of the water schemes like in Melka Belo woreda one on spot spring is damaged that can increase concentration of people and livestock on the remaining water points / ponds,
- The water can be lost in the form of evaporation and infiltration.

If there is no adequate rain fall of the main rain and the existing water in the ponds depilate, **91,830 people** (in Midhaga Tola, Mayu Muluke, Kumbi, Gursum, Chinaksen and Babile woredas) that have chronic water problem will be at risk of acute water shortage.

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Table 2: Woredas with chronic water problem that needs close monitoring and support

No.	Name of woreda	Number of PA	Number of population at Risk	Name of kebeles at Risk
1	Midhaga Tola	6	33,094	Bilisuma, Kufa, Negaya Midhaga, Roba, Ibiro Musa and Midhaga Kebeles
2	Gola Oda	2	8,000	Mino, Hariro, Gurati etc
3	Kumbi	8	15,000	
4	Mayu Muluke	5	15,500	Goro Negaya, Kile, Chalo, Hararse and Mojo wolday
5	Gursum	5	8,500	Borale zone
6	Chinaksen	3	7,203	Turanod, Hosale and Uswayine
7	Babile	1	4,533	Derar Arba
	Total	30	91,830	

Note: 30 PA's in 7 woredas needs close monitoring

Recommendation

- There is no WASH related emergency for the time being, however, in chronically water shortage areas like Midhaga Tola, Mayu Muluke, Kumbi and Babile woredas of east Hararge zone needs close monitoring of the water availability after the stop of the main rainfall. Even though, the ponds have harvested adequate water for the time being; due to heavy rainfall, the ponds capacity may have reduced due to siltation,
- There are 262 non functional) in East Hararge zone which is about 24% of the total water schemes, aggravating the shortage of potable water supply in the zone:
- Most of the non functional water schemes that can be maintained at woreda level are due to shortage of budget. Most of the water schemes don't have strong WASHCOs. Water schemes that don't have WASHCOs are suffering from lack of budget for maintenance. But Water schemes which have strong WASHCO, collect water fees

- and deposited in the bank account which they can use for maintenance and expansion works. Hence, establishing and strengthening of WASHCOs for each water schemes will ensure the ownership/ functioning of the water schemes on sustainable basis.
- For short term response; financial support is required for maintenance of the non function water schemes.
 - According to the report from zone water offices, 51.3% people in east Hararge zone are using unprotected water sources, mainly runoff water hence, to treat the water at household level, supplies of water treatment chemicals are required.
 - About **91,830 people** in Midhaga Tola, Babile, Gola Oda, Kumbi, Gursum, Mayu Muluke and Chinaksen woredas need close monitoring.
 - Speedup of the completion of the ongoing water supply projects for instance, in Gursum, Chinaksen, Babile and Gola Oda will contribute to alleviate the water shortage,
 - For AWD prevention and control, strengthen the taskforce at all level and support them with supply and budget.

West Hararge Zone

Key findings

- In west Hararge zones, the onset of the ‘belg’ rain was earlier different from the last 3 years and the rainfall pattern (distribution and amount) was good.
- The existing ponds in Boke, Burka dimtu, Daro Labu, Hawi Gudina and Mieso woredas are harvested adequate water which can serve for more than two months after the stop of rainfall. In Messela and Tulo woreda of West Hararge zone, increases in the yield of springs are reported. Generally the quantities of water availabilities for human and livestock consumptions are increased.
- In contrary, in some woredas (Mieso, Boke, Daro Labu) heavy rainfall was reported in April, which have resulted to damage of large ponds and existing water schemes.
- There is no WASH related emergency for the time being, however, in chronically water shortage areas like Daro Labu, Hawi Gudina, Boke, Burka Dhimtu and Mieso woredas of west Hararge zone needs close monitoring of the water availability after the stop of the main rainfall. Even though, the ponds have harvested adequate water for the time being;
 - ✓ Due to heavy rainfall, the ponds capacity may have reduced due to siltation,
 - ✓ Runoff flood has damaged some of the ponds and water schemes in Mieso and Boke woredas that can increase concentration of people and livestock on the remaining water points / ponds,
 - ✓ The water can be lost in the form of evaporation and infiltration.

- ✓ If there is no adequate rainfall of the main rain and the existing water in the ponds deplete, **105,002 people** in 5 woredas (Daro Labu, Boke, Burka Dimtu, Mieso and Hawi Gudina) that have chronic water problem will be at risk of acute water shortage.
- There are about 512 water schemes (409 functional and 103 non functional) in west Hararge zones. The non functional water schemes accounts to 20% of water the total water schemes. The non functional water schemes have aggravated the shortage of potable water supply in the zone:
 - Most of the non functional water schemes that can be maintained at woreda level are due to shortage of budget. Most of the water schemes don't have strong WASHCOs. Water schemes that don't have WASHCOs are suffering from lack of budget for maintenance. But Water schemes which have strong WASHCO, collect water fees and deposited in the bank account which they can use for maintenance and expansion works. Hence, establishing and strengthening of WASHCOs for each water schemes will ensure the ownership/ functioning of the water schemes on sustainable basis.
 - For short term response; financial support is required for maintenance of the non function water schemes.
 - According to the report from zone water office, 54.3% people in west Hararge zone are using unprotected water sources mainly runoff water, hence, supplies of water treatment chemicals are required to treat the water at household level.

- Speedup of the completion of the ongoing water supply projects in the zone will contribute to alleviate the water shortage.
- On the other hand, there is no school or health institutions closed due to WASH related emergency.
- Currently, there is no unusual water and sanitation related disease outbreak in the zone but still communicable disease outbreak like AWD is the threat in the zone. One AWD case from Anchar woreda is treated in East Showa zone, Fentale woreda.

WASH

This year the rainfall pattern (amount, distribution and duration of onset) was good. The belg rain was started earlier than the other year, in some places the amount is above normal. In April and May there was heavy rainfall (above normal) that has resulted to damage to water schemes in Mieso, Boke and Oda Bultum woredas. In areas that have chronic water problems, ponds have harvest adequate water that could last for more than two months after the end of the rainy season. The belg rain has improved the quantity of water availability. On the other hand, According to the report from zone water offices, 54.3% of people in west Hararge zone are using unprotected water sources, mainly runoff water. In the visited woredas like: Mesela, Tulo, Guba Koricha and Mieso water born and water scarce disease like skin disease, eye disease and diarrhea diseases are among the ten top disease. Hence, to treat the water at household level, supplies of water treatment chemicals are required.

Based on information obtained from zone water office, there are about 512 water schemes of which 409 functional and 103 are non functional due to low yield and mechanical problems. The non functional water schemes accounts to 20% of the total water schemes, which have

aggravated the shortage of potable water supply in the zone. Most of the mechanical problems that can be maintained at woreda level are due to shortage of budget. The majority of the water schemes don't have strong WASHCOs. Water schemes that don't have WASHCOs are suffering from lack of budget for maintenance. But Water schemes which have strong WASHCO, collect water fees and deposited in the bank account which they can use for maintenance and expansion works. Hence, establishing and strengthening of WASHCOs for each water schemes will ensure the ownership/ functioning of the water schemes on sustainable basis. The summaries of the status of the existing water schemes in West Hararge zone are summarized in the table below.

Table 3: Summaries of the status of existing water Schemes in West Hararge zone

No.	Type of scheme	Functional	Non functional	Total
1	Motorized BH	76	25	101
2	SW with hand pump	60	8	68
3	Hand dug well	74	21	95
4	Protected on spot spring	168	48	216
5	Spring with distribution system	31	1	32
	Total	409	103	512

For the time being, there is no WASH related emergency, however, there are pocket areas in Boke, Burka Dimtu, Daro Labu, Hawi Gudina and Mieso woredas of west Hararge zone that have chronic water shortage needs close monitoring of the water availability after the stop of the main rainfall. Even though, there was adequate rainfall and there are water in the ponds for the time being, the pond water may not last longer due to:

- The ponds capacity may have reduced by siltation as a result of heavy rainfall. This needs further investigation.
- Runoff flood has damaged some of the ponds like in Mieso, Boke and Oda Bultum woredas that can increase concentration of people and livestock on the remaining water points / ponds,
- The water can be lost in the form of evaporation and infiltration.

If there is no adequate rainfall of the main rain and the existing water in the ponds deplete, **105,002 people** in 5 woredas (Daro Labu, Boke, Burka Dimtu, Mieso and Hawi Gudina) that have chronic water problem will be at risk of acute water shortage.

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Table 4: Woredas with chronic water problem that needs close monitoring and support

No.	Name of woreda	Number of PA	Number of population at Risk	Name of kebeles at Risk
1	Daro Labu	11	30,173	Sakina, Kurfa Wachu, Biyo Oda, Tamsaye Tubo, Haro Adi. Haroresa Huluko, Boyido, Ijarti, Ido Jalata, Kurkura and Haniso
2	Boke	11	27,122	Burka Oda, Dimitu, Haro Gurati, Burka Ramis, Tayife, Tadacha Hatansa, Kurfa Galma, Mata Arba, Anub, Midhab and Arara
3	Burka Dimtu		21,169	
4	Mieso	2	4,038	Itsa Roro and Buri Mulu
5	Hawi Gudina	5	22,500	
	Total	29	105,002	

Note: **105,002 people** in 5 woredas needs close monitoring support

On the other hand there is no school or health institutions closed due to WASH related emergency, however, the schools and Health institutions in the affected kebeles are victims of the prolonged drought that leads to increased student drop outs and interrupted regular class attendance of students.

AWD – in West Harerege

Currently, there is no unusual water and sanitation related disease outbreak in the zone but still communicable disease outbreak like AWD and others are also the threat in the zone due to low water coverage and poor hygiene practices. The prevalence of water and sanitation related disease is among the ten top diseases.

Even though, in most of the woredas at community level hygiene awareness raising activities are underway by HEWs, emergency coordination committee established during AWD outbreak from the

zonal level to kebele level are weakened. At woreda level the capacities to deal with the epidemic in terms of drugs, logistics and finance in most words are inadequate.

Recommendation

- There is no WASH related emergency for the time being, however, in chronically water shortage areas like Burka Dhimitu, Boke, Daro Labu, Hawi Gudina and Mieso woredas of west Hararge zone needs close monitoring of the water availability after the stop of the main rainfall. Even though, the ponds have harvested adequate water for the time being; due to heavy rainfall, the ponds capacity may have reduced due to siltation,
- There are 103 non functional) water schemes in west Hararge zone which is about 20% of the total water schemes, aggravating the shortage of potable water supply in the zone:
 - Most of the non functional water schemes that can be maintained at woreda level are due to shortage of budget. Most of the water schemes don't have strong WASHCOs. Water schemes that don't have WASHCOs are suffering from lack of budget for maintenance. But Water schemes which have strong WASHCO, collect water fees and deposited in the bank account which they can use for maintenance and expansion works. Hence, establishing and strengthening of WASHCOs for each water schemes will ensure the ownership/ functioning of the water schemes on sustainable basis.
 - For short term response; financial support is required for maintenance of the non function water schemes.
 - According to the report from zone water offices, 53.3% people in east Hararge zone are using unprotected water sources,

mainly runoff water hence, to treat the water at household level, supplies of water treatment chemicals are required.

- About **105,002 people** in 5 woredas needs; Burka Dhimitu, Boke, Daro Labu, Hawi Gudina and Mieso woredas need close monitoring.
- Speedup of the completion of the ongoing water supply projects in the zones will contribute to alleviate the water shortage,
- For AWD prevention and control, strengthen the taskforce at all level and support with supply and budget are required.

6. Annexes

6.1 Hazards and Number of Population at Risk

Name of Woreda	Emergency type and number of population			Remark
	Hazard	Affected population	At risk population	
East Hararge zone				
Midhaga Tola	Drought		33,094	
Gola Oda	Drought		8,000	
Kumbi	Drought		15,000	
Mayu Muluke	Drought		15,500	
Gursum	Drought		8,500	
Chinaksen	Drought		7,203	
Babile	Drought		4,533	
Sub Total			91,830	
East Hararge zone				
Daro Labu	Drought		30,173	
Boke	Drought		27,122	
Burka Dimtu	Drought		21,169	
Mieso	Flood		4,038	Earthen dam damaged with a flood
Hawi Gudina	Drought		22,500	
Sub Total			105,002	
Total			196,832	



6.2 Status of water Schemes and water coverage in East Hararge zone

No .	Name of woreda	Type of Schemes												Total	Water coverage (%)
		Motorized Borehole		SW with hand pump		HDW with hand pump		on spot Spring protected		Spring with distribution		Total			
		Fun.	Non. Fun.	Fun.	Non. Fun.	Fun.	Non. Fun.	Fun.	Non. Fun.	Fun.	Non. Fun.	Fun.	Non. Fun.		
1	Haramaya	3		56	3	66	2	2		5		132	5	137	73.8
2	Deder	3			1	8	5	17	12	13	9	41	27	68	43.5
3	Goro Gutu	4	1	5	1			23	2	23	3	55	7	62	37.8
4	Meta	2		7	7			23	5	7		39	12	51	36.4
5	Kersa	10	4	55	14	11	4	3	3			79	25	104	77.1
6	Badeno	4					2	17	18	14	14	35	34	69	42.5
7	Melka Belo	1						5	1	10	2	16	3	19	23.7
8	Gola Oda	11	2	6	2							17	4	21	80.3
9	Gursum	1	4	66	4	43	3	2		6		118	11	129	56.4
10	Kurfa Chale							2		31	13	33	13	46	71.2
11	Jarso	3	1	6				3	1	8	1	20	3	23	17.4
12	Midhaga Tola	1	4									1	4	5	33.7
13	Meyu Muluke	4	1	2	2							6	3	9	45.1
14	Girawa	4	3			7	5	2	4	2		15	12	27	40.0
15	Fedis	14	2	8	7							22	9	31	55.4
16	Chinaksen	6	11	44	11	27	1	2				79	23	102	65.7
17	Babile	2		54	43	57		1	2	1		115	45	160	84.0
18	Kombolcha	1		4	9		6	12	4	2	3	19	22	41	34.8
19	Kumbi	0	0	0	0	0	0	0	0	0	0	0	0	0	0.0
	Total	74	33	313	104	219	28	114	52	122	45	842	262	1,104	48.7

6.3 Status of water Schemes and water coverage in West Hararge zone

No.	Name of woreda	Type of Schemes													Water coverage (%)
		Motorized Borehole		SW with hand pump		HDW with hand pump		on spot Spring protected		Spring with distribution		Total		Total	
		Fun.	Non. Fun.	Fun.	Non. Fun.	Fun.	Non. Fun.	Fun.	Non. Fun.	Fun.	Non. Fun.	Fun.	Non. Fun.		
1	Chiro	6		2	1	11	8	17	1			36	10	46	48.0
2	Mieso	20	4									20	4	24	48.6
3	Oda Bultum	3	2	11		33		24				71	2	73	54.5
4	Boke	8	2									8	2	10	21.8
5	Habiro	8	8	25	7	21	12	13	16			67	43	110	46.4
6	Daro Labu	15	1	8		6		13				42	1	43	43.4
7	Tulo	2		9		1		20	1	11	1	43	2	45	40.5
8	Doba	2	2			1	1	26	6			29	9	38	21.8
9	Mesela							6	13	18		24	13	37	37.3
10	Gamachis	6	1	5				18				29	1	30	33.7
11	Guba Koricha	3	2					6		2		11	2	13	24.5
12	Anchar	2				1		24	9			27	9	36	41.2
13	Hawi Gudina	1	3									1	3	4	20.4
14	Burka Dhimtu							1	2			1	2	3	21.6
	Total	76	25	60	8	74	21	168	48	31	1	409	103	512	45.7

Source: West Hararge zone water office

6.4 Budget Breakdown for Water rationing for People at Risk in East and West Hararge zones

No	Wereda	Population at Risk	Quantity of water per person/day (Lit)	Quantity of water Required /day (m³)	Cost of water per (m³)	Total Qty & Cost of water for 3 Months		Water Truck Required	Unit cost for Water Truck per Day (ETB)	Total Cost for Water Truck for 3 months (ETB)	Total Cost for 3 months (ETB)
						Qty (m³)	Cost (ETB)				
1	East Hararge zone										
	Midhaga Tola	33,094	5	165	7	14,892	104,246	5	1,500	675,000	779,246
	Gola Oda	8,000	5	40	7	3,600	25,200	1	1,500	135,000	160,200
	Kumbi	15,000	5	75	7	6,750	47,250	2	1,500	270,000	317,250
	Mayu Muluke	15,500	5	78	7	6,975	48,825	2	1,500	270,000	318,825
	Gursum	8,500	5	43	7	3,825	26,775	1	1,500	135,000	161,775
	Chinaksen	7,203	5	36	7	3,241	22,689	1	1,500	135,000	157,689
	Babile	4,533	5	23		2,040	14,279	1	1,500	135,000	149,279
	Sub Total	91,830		459		41,324	289,265	13	1,500	1,755,000	2,044,265
2	West Hararge zone										
	Daro Labu	30,173	5	151	7	13,578	95,045	5	1,500	675,000	770,045
	Boke	27,122	5	136	7	12,205	85,434	4	1,500	540,000	625,434
	Burka dhimtu	21,169	5	106	7	9,526	66,682	3	1,500	405,000	471,682
	Mieso	4,038	5	20	7	1,817	12,720	1	1,500	135,000	147,720
	Hawi Gudina	22,500	5	113	7	10,125	70,875	3	1,500	405,000	475,875
	Sub Total	105,002	5	525	7	47,251	330,756	16	1,500	2,160,000	2,490,756
	Total	196,832				88,574	620,021	29	1,500	3,915,000	4,535,021

Other Expenses related to Water Tankering

No	Description	Unit	Quantity	Unit Cost per day (Birr)	Total Cost for 3 months
	Per dime for water distributors	No.	29	70	182,700
	Fuel	Liter	100	12	108,000
	Lubricants (3% of fuel cost)	LS			3,240
	Sub Total				293,940

Grand Total**4,828,961 Birr**

Discussion

It was about one month ago, that a multi-agency group has gone out to the different zones and weredas of Oromia region to conduct Belg or seasonal productivity in these localities. This assessment encompasses water supply, nutrition, and health-related issues particularly, that of children under five years of age and the population at large. Indeed, rain fall and climate, the cultivated and the uncultivated land including the agricultural in-puts and many other factors were taken into consideration in order to come-up with sound findings leading to an effective and efficient action. On the other hand, the health part focused on the status of vaccination and nutrition, plus school drop-outs in the child hood population. Even though, the assessment was rapid in nature due to time constraint, physical observation of the surrounding vegetation, consultation and discussion with local administrative officials and experts of different ministry branches plus review of the available documents were assessed in all the visited weredas by the assessment team. In this regard, East and West Harage zones and the more than 20 weredas, which are prone to run risk of being affected by lively hood- hazards, were included. The findings of their assessment showed that they have so many factors characterizing all of them in common. The highly hilly and slippery land feature covered with big stones coupled with backwarded farming system obviously, results in low productivity of cereals and tuber crops which does have an indirect negative impact in market availability of sources food items and subsequent nutritional deficiency in children or for all members of a house hold. Here, the important thing is that the problem is continuous, and found to be so, according to the explanation made by the local people. Also, when ever it rains heavily, any harvest in the field will be damaged. The heavy rain which destroyed the potato of the present *Belg season* in the high land of the assessed weredas is the testimony of such event. Again seeing this problem as a public health person and in terms of health, it is, its relation to nutrition and to heath, above all in the vulnerable group of the population. According to the physical observation of the road sides of the visited weredas scarcity

and in most cases absolute absence of trees is seen. Then, knowing the close relationship that exists between rain and availability of water in an area for adequate and safe supply of the later related to water-borne diseases and sanitary health problems the situation in the assessed weredas needs due solution. The information described in the table below about children with malnutrition in the OTPs, availability of water supply in the different localities, plus students drop-outs due to lack of water or substituting their mothers to fetch water or to take care of their youngsters is some of the illustrations for the health and health-related problems existing in the assessed weredas.

In summary, the assessment team believes that the factors contributing to the health and health-related situations of these weredas where *Belg seasonal* assessment took place in the weredas of the two zones were almost similar. These factors are also found intertwined seeming to make difficult enough to give immediate solutions. However, the involvement of NGOs, like CARE, which provides crops and financial support for safety net for food security purpose at household level and the support of IMC to tackle the nutritional problem of the childhood population need to be wisely utilized and strengthened for the future. Indeed, if possible AWD, and mainly Measles epidemics could be minimized or avoided within short period of time. The effort which is underway to overcome nutritional problem by feeding children with locally available food items and in collaboration with an organization called Community based nutrition (CBN) needs to be strengthened further more. Thus, if the community is very well sensitized and mobilized to stand on the front line against its own problems and get support from all corners it is possible to bring-about changes.

Recommendation

- Conservation and planting trees,
- Maintenance of water Schemes,
- Improvement of Immunization and family planning services,
- Improvement of Water supply,
- Undertake Extensive Health Education to the community via Health Extension workers (HEWs),
- Strengthen of inter-sectoral coordination and collaboration,
- Emergency Preparedness and response shall in place
- Scale -up of Water coverage,
- Scale-up of Latrine and proper utilization of it,
- A Strong Inter-Regional collaboration has to be emerged in order to address on-going emergency issues specifically and emergency preparedness and generally between regions has to be made

Conclusion

- No budget allocated for epidemic preparedness and response at Woreda and Zone level. There is inadequate contingency stock of drugs and medical supplies. Thus, Woreda and Zone health office should incorporate budget allocation for epidemic preparedness and response on their annual plan.
- For AWD prevention and control, strengthen the taskforce at all level and support them with supply and budget.
- Due to poor food security situation at house hold level and absence of supplementary feeding readmission and new SAM are increasing. Therefore, the team recommends that beside UN agencies and NGOs, it would be better if the regional government participate in provision of supplementary feeding for screened Pregnant & Lactating mothers and malnourished children.
- There is meager management of data at woreda and zone level. Thus, It would be better, if the regional health bureau give attention for data management and provide supportive supervision to zonal and woreda health offices.
- Contingency stock of drugs and medical supplies are not Adequate/ available for some epidemic prone diseases at zone and woreda level. Therefore, it is recommended to secure this contingency stock in advance from decentralization point of view.
- The most important thing is also Family planning must be a major agenda for both zones
- The major low land parts of the visited zones are use unprotected water sources mainly ponds so that people are at risk of water driven health problems.
- To improve/ maintain the yields of water schemes (especially shallow wells and springs), attention should be given to soil and water conservation (watershed management). Otherwise, the fate of the ongoing water developments and the rest water schemes will be dry after short period of time.
- Strict close monitoring is required for the population at risk of WASH problems in all visited zones.

- With the onset of rain as community are going to use the runoff water, supply of water purification chemicals are required.
- Drilling of deep and shallow wells can alleviate the existing problem of drinking water so that he speed up of the completion o f the ongoing water supply projects in the zones/woredas shall speed up to contribute and alleviate the water shortage.

List of Participant

- Mrs. Almaz Girmay – DRMFSS-Team Leader
- Mr. Getaneh Abreha – FMOH/EHNRI – PHEM
- Dr. Gabissa Tollosa – ORHB,
- Mrs. Zenebech Bekele – WFP,
- Mr. Dinku Gutema, : Oromia-Water Bureau
- Mr. Chemedi Nurgi– UNICEF
- Mr. Negusu Worku – WHO
- Mr. Reshide Abdi-USAID,
- Mr. Wubeshite Genene – CARE – East Harerege



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Humanitarian Needs assessment- Health Sector: Regional and Zonal level Questionnaire

Interviewer name _____		Institution: _____	
Interview Date: (dd) ____ / (mm) ____ / 2010		Region: _____	
Main contact at this location:	Name: _____	Position: _____	Tel: _____
1. COORDINATION			
A. Is the RHB actively participates in the Regional DRM taskforce?		Yes ☐	No ☐
B. Is there a functional multisectoral coordination forum for the health sector?		Yes ☐	No ☐
C. Are all relevant government, NGOs and UN agencies represented?		Yes ☐	No ☐
D. Frequency of regular meeting? _____			
2. Public Health emergency Management			
A. Does the RHB developed multi disease EPRP?		Yes ☐	No ☐
B. Is there a PHE preparedness fund?		Yes ☐	No ☐
C. Does the RHB has accessible PHE response fund?		Yes ☐	No ☐
D. Is there a dedicated PHEM unit/team?		Yes ☐	No ☐
E. How many staff are in the team?		Yes ☐	No ☐
F. Are PHEM team members trained on			
i. PHEM – concept and operational framework Yes ☐ No ☐			
ii. IDSR- Yes ☐ No ☐			
iii. EPRP preparation- Yes ☐ No ☐			
iv. Outbreak management of major epidemic prone diseases Yes ☐ No ☐			
G. Is there stock of			
i. Meningitis vaccine- Yes ☐ No ☐ ; if yes specify amount			
ii. Measles Vaccine - Yes ☐ No ☐ ; if yes specify amount			
iii. Drugs : Oily CAF - Yes ☐ No ☐ ; if yes specify amount			
Doxycycline- Yes ☐ No ☐ ; if yes specify amount			
Ringer lactate- Yes ☐ No ☐ ; if yes specify amount			
ORS - Yes ☐ No ☐ ; if yes specify amount			
H. State Major challenges in PHEM: _____			
I. List Major areas of support from Federal and Health Partners: _____			

**Humanitarian Needs assessment- Health Sector : Woreda level Questionnaire**

Interviewer name_____		Institution:_____		
Interview Date: (dd)_____ / (mm)_____/2010		Region:_____ Zone:_____ Woreda_____		
Main contact at this location:	Name:_____ —	Position:_____ —	Tel:_____ —	
SECTION I: SOCIO- DEMOGRAPHIC PROFILE				
1.1. Woreda total population:	M:_____ F:_____	Under :5_____	Above5:_____	
1.2. Special Population (if any):	Pastorals_____	Refugees_____ —	IDPs_____ Migrant Workers_____	
SECTION II: HEALTH PROFILE				
2.1. Morbidity and Mortality (List top 5 causes of Morbidity and Mortality)				
a. Morbidity		b. Mortality		
2.2. Number of AWD cases/deaths since Sep 2009-May 2010 (Meskerem – May 2002)				
a. AWD	Sep: cases_____	deaths_____	Feb: cases_____	deaths_____
	Oct: cases_____	deaths_____	Mar: cases_____	deaths_____
	Nov: cases_____	Deaths_____	Apr: cases_____	deaths_____
	Dec: cases_____	deaths_____	May: cases_____	deaths_____
	Jan: cases_____	deaths_____	Jan: cases_____	deaths_____
b. Malari a	Sep: cases_____	deaths_____	Feb: cases_____	deaths_____
	Oct: cases_____	deaths_____	Mar: cases_____	deaths_____
	Nov: cases_____	Deaths_____	Apr: cases_____	deaths_____



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				cases_____	
	Dec: cases_____	deaths_____	May: cases_____	deaths_____	
	Jan: cases_____	deaths_____	Jan: cases_____	deaths_____	
c. Meningitis	Sep: cases_____	deaths_____	Feb: cases_____	deaths_____	
	Oct: cases_____	deaths_____	Mar: cases_____	deaths_____	
	Nov: cases_____	Deaths_____	Apr: cases_____	deaths_____	
	Dec: cases_____	deaths_____	May: cases_____	deaths_____	
	Jan: cases_____	deaths_____	Jan: cases_____	deaths_____	
d. Measles	Cases_____		Deaths:_____		

2.3. Any unusual event (disease) if yes: specify the disease _____
time of occurrence _____, Cases _____ Death _____

2.4. Is there any ongoing outbreak? YES _____ NO _____
If yes, specify the type of disease _____
Number of cases _____ Deaths _____ (specify the time period) _____

2.5. Disease Control and Prevention

B. Report the coverage of the following services during the year 2001EFY (Sep 2008-Aug 2009)

Measles Vaccination in u-1:	_____
Pentavalent Vaccination in u-5:	_____
ITN Coverage:	_____
IRS:	_____

C. Fill in the table “YES” or “No” health services rendered by HF in the Woreda

GCS: General Clinic Service; **MCH:** Mother & Child Health; **ES:** Emergency Surgery; **EOC:** Emergency Obstetric Care; **MH:** Mental Health; **Nut:** Nutrition; **EPI:** Expanded Program for Immunization; **FP:** Family planning; **ANC:** Antenatal Care; **Lab:** Laboratory Service

GCS	MCH	ES	EOC	MH	Nut	EPI	FP	ANC	Lab

D. Supplies :

i. Essential drugs are available for : (Enough to treat cases for 6 months)

A W D	Yes	No	Malaria	Yes ☐	No ☐	Meningitis	Yes ☐	No ☐	Measles	Yes ☐	No ☐
	☐	☐									

ii. Intravenous fluids : Ringer Lactate: Yes ☐ No ☐

iii. ORS: Yes ☐ No ☐

iv. Consumables : Syringes, Gloves:						Yes ☐		No ☐			
Main Shortage (if any): Specify											
E. Are case management guidelines available for											
A W D	Yes ☐	No ☐	Malari a	Yes ☐	No ☐	Meningiti s	Yes ☐	No ☐	Measles	Yes ☐	No ☐
F. Human resource:											
Type of Health Workers Available						Number					
i. Health Officers:											
ii. Nurses:											
iii. Lab technicians:											
iv. EHWS											
Number of trained health workers by program											
Program		Number/ Type of trained health workers									
PHEM											
IDSR											
EPI											
Malaria											
Nutrition											
G. PHEM											
i. Is there a multisectoral PHEM coordination forum						Yes ☐				No ☐	
ii. Is there an EPRP						Yes ☐				No ☐	
iii. If yes, is the plan operational?						Yes ☐				No ☐	
If no Why? _____											
H. Information Management											
i. Woreda IDSR timeliness rate for the last 6 months:						%					
ii. Are data collected from HFs being analyzed (trend line....)						Yes ☐				No ☐	
iii. Major constraints in data analysis and reporting											
I. Water Supply											
i. Percentage of Woreda population using improved water source (protected spring, well, pipe line distribution, HH connection) for human consumption (drinking, cooking and hygiene)= % _____											
ii. Percentage of Woreda population getting water from treated source (e.g pipe line, HH connection...)= % _____											
SECTION III: RISK FACTORS											



Belg – Multi – Sectoral Humanitarian needs assessment from June 22 – July 12, 2010: By Negusu Worku GEBRMICHAEL [negusuw@et.afro.who.int]

Diseases	Risk factors	Exist (Yes/No)
3.1.Malaria	Altitude < 2000m	
	Malaria Endemic	
	Presence of malaria breeding site	
	Interrupted or potentially interrupting rivers	
	Unprotected irrigation	
3.2.Meningitis	Is the woreda in the meningitis belt	
	Was there Meningitis epidemic in the last three years (If yes specify year and month)	
	If yes : number of people vaccinated	
3.3.AWD	History of AWD epidemic in the last three years	
	If yes specify year and month	
SECTION IV: Health care financing		
4.1. List the type of service which are provided freely to the community currently		
a. _____		
b. _____		
c. _____		
d. _____		
e. _____		
4.2.Is there a waiver/exemption system for people who cannot afford to pay for health services		
If so, what is the criteria and who decides on the selection		
4.3.Which services are covered by the waiver system		
a. _____		
b. _____		
c. _____		
d. _____		
e. _____		
f. _____		
4.4.Is there Social insurance		
Health Partners working in the Woreda		
Name	Program	
Assessment team Major observation		

**Section V: Nutritional Status at woreda level 2010 (2002)**

Woreda	Total SAM Cases	Number of TFP (OTP/SC) reporting	Number of SC .	Number of OTP.	Therapeutic Supplies enough (RUTF) Y/N	Children Discharged from TFP received SF
	Jan_____	Jan_____	Jan_____	Jan_____		
	Feb_____	Feb_____	Feb_____	Feb_____		
	Mar_____	Mar_____	Mar_____	Mar_____		
	Apr_____	Apr_____	Apr_____	Apr_____		
	May_____	May_____	May_____	May_____		
	Jan_____	Jan_____	Jan_____	Jan_____		
	Feb_____	Feb_____	Feb_____	Feb_____		
	Mar_____	Mar_____	Mar_____	Mar_____		
	Apr_____	Apr_____	Apr_____	Apr_____		
	May_____	May_____	May_____	May_____		
	Jan_____	Jan_____	Jan_____	Jan_____		
	Feb_____	Feb_____	Feb_____	Feb_____		
	Mar_____	Mar_____	Mar_____	Mar_____		
	Apr_____	Apr_____	Apr_____	Apr_____		
	May_____	May_____	May_____	May_____		
	Jan_____	Jan_____	Jan_____	Jan_____		
	Feb_____	Feb_____	Feb_____	Feb_____		
	Mar_____	Mar_____	Mar_____	Mar_____		
	Apr_____	Apr_____	Apr_____	Apr_____		
	May_____	May_____	May_____	May_____		
	Jan_____	Jan_____	Jan_____	Jan_____		
	Feb_____	Feb_____	Feb_____	Feb_____		
	Mar_____	Mar_____	Mar_____	Mar_____		
	Apr_____	Apr_____	Apr_____	Apr_____		
	May_____	May_____	May_____	May_____		

NB: SF= Supplementary Food, SC = Stabilization Center, OTP= Outpatient Therapeutic Program, SAM = Severe Acute Malnutrition RUTF= Ready to Use Therapeutic Food /Plump Nut, F75 and F100/ NS =Nutrition Survey RA =Rapid Assessment

Summary : Emergency Health and Nutrition needs assessment requirements/Needs/ 2010

Intervention	Activities	Woreda	Total estimated Beneficiaries	Quantities	Estimated Cost



Photo Source: Negusu Worku/WHO/EHA

One of the In-depth Interview and Discussing the Current Multi-Agency Situation in
Girawa District, East Harerege Zone, Oromia Region

End

